

MEQ 1

You are a CL psychiatrist. Jerry McIntosh is a 53 y/o secondary school teacher admitted under the medical team for new onset of neurological issues. He has difficulty walking, memory difficulties, and he has not been sleeping, with irritability and grandiose ideas. He believes he will cure the world of hunger through a new substitute food he has invented. Jerry is a homosexual and has been HIV for 30 years. The medical team believe that he has become manic and are requesting your assessment

1.1 Outline your approach to the assessment of Jerry including a cognitive assessment (10 marks)

1. Obtain information/history taking

- Medical team – determine progression and sequelae of HIV disease
 - Patient consenting to psychiatric review
 - HIV treatment information – has he been adherent
 - Planned treatment and expected duration of admission, prognosis, other medical conditions
 - Current medications – recent changes, side effects, interactions, steroids
 - Previous investigations
 - Brain imaging in context of brain related HIV changes and any acute intracerebral changes
 - Bloods – HIV viral load, CD4
- Family – determine premorbid functioning
 - Current living situation
 - Next of kin, EPOA
 - Significant relationships
 - Employment – when last worked, current financial status
 - Recent functioning - need for support, critical incidents
 - Substance abuse hx
- Psychiatric History
 - Previous contact with MH specifically and diagnoses of BPAD

2. Cognitive/physical and MSE assessment

- MSE – consider BPAD, psychosis
- Screen for delirium – attention concentration, diurnal disturbances. CAM
- Frontal lobe testing – set shifting, executive functioning
- MMSE, MOCA - HIV related dementia
- Physical exam – neuro exam, vital signs – consider signs of CVA, infection

3. Risk assessment

- Risk to self – clarify from MSE any risks to self of self harm or misadventure
- Risk to others – history of aggression
- Risk of absconding – in the context of possible confusion
- Risk to reputation – current symptoms of mania, impact at work

You diagnose Jerry with HIV dementia. Jerry's partner of 15 years, Bill is concerned about Jerry's recent decline. Bill comes to you and would like to know what is happening. Bill is worried about the impact of Jerry's decline on his work and on their relationship

1.2 Outline your response to Bill

• Consent and confidentiality and ethical concerns

- Clarify whether Jerry has consented for us to talk to Bill
- Clarify capacity and substitute decision making and consider EPOA and AHD

- **Clarify Bill's concerns and any relevant past experiences**
 - Validate his concerns
- **Information**
 - Clarify knowledge of diagnosis and invitation to provide further explanation
 - Psychoeducation regarding Jerry's diagnosis, treatments, prognosis
 - Clear explanation to Bill that HIV dementia is a progressive illness and while treatments will focus on maximizing function and quality of life, his dementia will continue to worsen
 - Psychoeducation regarding the role of psychiatry and management of psychiatric symptoms as a result of HIV dementia
- **Specific response on work and relationship concerns**
 - Clarify relationship dynamics and his concerns
 - Clarify work related concerns? financial burden
 - Idea of Jerry's current health and level of functioning – care needs, home supports
- **Liaison**
 - Pass on Bill's concerns
 - Liaison with social worker and OT – functional Ax
 - Offer to be involved in family meeting including treating medical team
- **Referral and support**
 - To counselling for Bill – specifically for grief, carer's burden
 - Counselling around role transition
 - Encouraging Bill to access support from family/friends and community HIV support groups

MEQ 2 (8.1 RANZCP) 25 marks

You are working as a consultant liaison psychiatrist in the CL service of a general hospital. You are contacted by your registrar on a medical ward with the message that the medical service would like to transfer a patient to the psychiatry service. Theo is a 25-year-old man who was admitted to the medical service three days ago after an overdose involving multiple substances, not all of which are known. He is proving difficult to manage on the medical ward due to his wandering, agitation, and occasional aggressive outbursts without physical violence. He does not make much sense when he talks and the medical team feel he is clearly psychotic and should be transferred to the inpatient psychiatry team.

2.1 Outline your approach to managing this situation (6 marks)

- Identify need for comprehensive assessment before consideration of transfer
- Medical teams concern and rationale for diagnosis of psychosis
- **Registrar's assessment – to corroborate rationale for diagnosis of psychosis**
 - History of presenting complaint
 - Past psychiatric and medical history
 - Past psychosis or mood dx
 - Seizure dx, Intellectual impairment/autism
 - Family history
 - Treatment history - past admissions, overdoses, ICU admission
 - Mental state exam
 - Substance use history – intoxication and withdrawal
 - Collateral information from family
 - Delirium screen
 - Review of documented behavior
- Current medication and behavioral management and response to this
- **Capacity and legal implications**
 - Is the patient currently under the MHA or emergency provisions of the guardianship act
- **Medical Issues and Ix**
 - Details of overdose and treatment/monitoring/need for ICU
 - Recent blood Ix, imaging, EEG and urinalysis (causes for delirium/hypoxia)
 - Current medications
- **Risk assessment and mitigation**
 - Risk to patient – current suicidal risk – thoughts, intent, plan
 - Risk to others – of aggression
 - 1:1 special
- Dynamic issues - staff countertransference and burnout
- System issues - lack of education, skills, clinical support in managing difficult behaviors
- **Support for the registrar**
 - Offer to see the patient
 - Offer to liaise with the consultant physician
 - Follow up discussion in supervision

Your registrar is able to tell you that the patient was brought in by the police after being found unconscious in his car. His blood alcohol on admission was 0.21% and he had a paracetamol level which was mildly elevated but not in the toxic range, although the exact time of ingestion was not known. Benzodiazepines and cannabinoids were also detected. On examination you find the patient to have fluctuating level of consciousness. He is disoriented, highly distractible and over-active with repetitive, semi-purposeful movements. He is unable to give much coherent history and talks of seeing little creatures in his room. His affect appears fairly cheerful and he tries to be cooperative. He is somewhat flushed and his chart indicates a mild pyrexia of 37.8 degrees.

2.2 What causes of the patient's condition should be considered and what other information (from any available source) would help support or exclude these potential causes? (10 marks)

Delirium: Complete delirium screening

□ Substance withdrawal/polypharmacy

- Current polypharmacy - opioids, anticholinergics, benzos, antipsychotics
- Withdrawal from alcohol/benzodiazepines/opioids – in view of elevated blood alcohol levels on admission
 - Urine drug screen
 - AWS
 - History of substance use and dependence

□ Infection – signs of delirium (disorientation, incoherent behaviour, visual hallucinations)

- Infective source – bloods, culture, CXR (aspiration), urine, vital signs

□ Serotonin syndrome/NMS - potential effects from polypharmacy

- Physical examination – myoclonus, tachycardia, fever, pupillary changes, hyperreflexia

□ Anticholinergic syndrome

- Physical examination - Flushing, dilated pupils, confusion, tachycardia

□ Encephalopathy/brain hypoxia/traumatic brain injury/seizure disorder

- Post ictal confusion, observed tonic/clonic/absence phenomena
- Brain imaging - evidence of trauma, CVA
- EEG
- Inflammatory markers, autoantibodies, ammonia level
- Lumbar puncture

Underlying psychiatric condition

- Active mood or psychotic disorder

Other - abnormal illness behaviour, malingering, intoxication on the ward

Your registrar is able to get in touch with Theo's mother. She tells the registrar that Theo has had problems for some years with alcohol and marijuana. As far as she knows he does not use heroin or amphetamines. His pattern of alcohol use is mainly one of bingeing every few weeks and, when he can afford it, he uses marijuana daily. However, he lost his job a few months ago and since then he has been experimenting with readily obtainable psychoactive substances such as panadeine forte which his father is prescribed for back pain. After Theo's overdose his mother noted that supplies of his father's medication for back pain and Parkinson's disease are also missing - the bottles are 'diazepam' and 'benzhexol'.

2.3 What is the most likely diagnosis (2 marks)

Delirium secondary to polysubstance intoxication and withdrawal, likely anticholinergic load

2.4 What advice about immediate management will you give your registrar? (7 marks)

- Ensure patient is not accepted for transfer at this time
- **Biological management**
 - Medication review and rationalization
 - Avoiding use of benzos or anticholinergics and anything that could worsen delirium
 - Considering low dose antipsychotic such as risperidone or haloperidol
 - More frequent, low doses
 - Treat any other underlying causes (electrolyte disturbances, pain, infections)
 - Ward environment

- Nursing – one to one special (focal nurse), regular documentation of cognition, orientation, behaviour
- Environment – low stimulus, ideally single room
 - Orientation board
 - Family photos and other personal effects (sensory aids, glasses)
 - Regular day-night cycle in the ward
 - Approach to patient – non-confrontational, re-orientation
- General supportive care – food/fluid/bowels chart
- Manage withdrawals using standardized scales and treatment regimes
- **Psychological**
 - Psychoeducation and reassurance about delirium – for patient, family
 - De-escalation information to nursing
 - Offer nursing in-service on delirium management
- **Liaison duties**
 - Communicate to medical team primary diagnosis that of delirium
 - Offer to be contacted by team and to arrange MDT meeting
 - Regular reviews /coordinated input
 - Continue revision of diagnosis
 - Consider referral to drug and alcohol services
- If not known clarify MHA status - not appropriate

MEQ 3 (8.1 RANZCP)

You are a child and adolescent psychiatrist. Ben is a 14 year old boy referred to you by his GP for an assessment. Ben has not attended school for 6 weeks now. Ben attributes his school refusal to needing to sleep. He finally sleeps around 3am. He wakes at 7am still feeling unrefreshed. Ben naps on and off from 5pm to 10pm. The GP has done some basic blood tests including FBC, UEC, LFT and TFT which were all normal. An examination revealed a healthy 14 year old with normal body weight, respiratory exam and GI exam. Vital signs were all normal.

3.1 Outline your approach to assessment (10 marks)

1. Multimodal, longitudinal collateral from multiple sources

- a. School report and observations to determine learning difficulties
 - i. ODD/Conduct problems/ADHD
 - ii. Learning problems/difficulty concentrating
- b. GP – past medical history, medication hx, family hx (predisposing factors)
- c. Parents – behavioural patterns at home, difference at home vs school

2. HPC

- a. Sleep-wake hx:
 - i. Sleep pattern, recent changes?
 - ii. Initial insomnia or waking throughout the night
 - iii. Sleep hygiene screen - Avoid screens before bed, caffeine intake, substance use, social media, gaming
 - iv. Management tried - medications? Sleep behavioural interventions

3. ABC Analysis

- a. Antecedence for refusal - Bullying, new school, new teacher, peer issues, academic stress
- b. Avoidance of social contact, sport, academic work,
- c. Behaviour when refusing - ?agitated, anxious, panic, parental response/boundaries/consequences
- d. ? Separation anxiety - and recent triggers
- e. Home factors - DV, abuse, substance use issues, parental MI, financial issues, recent bereavement

4. Screen for psychiatric diagnoses (contributing to sleep or refusal)

- a. General / social anxiety/ separation anxiety
- b. Mood disorder
- c. Trauma
- d. Substance use do

5. Risk assessment – self-harm, harm to others, risk of further deterioration (milestones), arrested development

You assess Ben. There are no psychological or neurovegetative features of depression. Ben has been having sleep issues for around one year, beginning following an episode of glandular fever. Other issues at home include fighting with his two younger siblings. His teachers report bullying at school for the past two months. Ben's school attendance and grades had been worsening over the past year.

3.2 List your differential diagnosis (5 marks)

1. Learning difficulties, developmental disorder (ASD, ODD, ADHD)
2. Mood disorder
3. Anxiety disorder (Social/Generalized/Separation)
4. Psychosocial factors (trauma, bullying, family dynamics)
5. Insomnia secondary to gaming use disorder/social media use disorder
6. Chronic fatigue syndrome

7. Specific phobia

Ben's parents are very worried about his sleep difficulties and ask what can be done to help. Ben's mother begs you to give him something to help, as she can't sleep a wink either, with Ben's noisy computer game playing. She has heard that 'valium' may help.

3.3 Outline your response to Ben's parents, including managing his insomnia (8 marks)

1. Acknowledge and validate her concerns and impact on family. Build therapeutic alliance, work in a collaborative manner
2. **Psychoeducation**
 - a. Psychoeducation around sleep difficulties in adolescents. Identify causal factors and emphasis likely multifactorial - Bullying, secondary gain, poor sleep hygiene, mental illness.
 - b. Importance of parental boundary setting
 - c. Diazepam
 - i. Tolerance, dependence, addictive potential
 - ii. Long term exacerbation of sleep issues
 - iii. Not indicated in paediatric insomnia
3. **Sleep hygiene:**
 - a. Emphasize importance of routine
 - b. Avoid screens before bed
 - c. Regular bedtimes, exercise, regular meal times
 - d. Avoid daytime napping
 - e. Reduce caffeine
 - f. Ideally use bedroom only for sleep
4. **Specific interventions:**
 - a. Sleep restriction therapy, Stimulus control therapy
5. **Pharmacological management** (if above fails): Melatonin rather than diazepam (off-label)
6. **Liaison** – Consider referral to paediatrician, GP

MEQ 4

You are a psychiatrist, specialising in veteran's mental health, and have been asked down to the emergency department by the eCAT clinician to assess Bill Anderson, a 67-year-old Vietnam veteran. He was brought in by police, following an altercation with the tram inspectors. Bill had reportedly become aggressive and threatening when asked for his ticket. Bill has been causing trouble in ED, shouting at co-patients that they are communists and demanding to leave. During the eCAT clinician's assessment, she observed that Bill appeared intoxicated, slurring his words and smelling of alcohol. He was unkempt and appeared not to have showered in days. Bill was not known to mental health services and would not provide details for a next of kin, so limited information is available for him.

4.1 Outline the barriers to assessing Bill (7 marks)

1. **Patient factors**
 - a. Risk of aggression, Uncooperative □ patient and staff safety at risk
 - b. Factors affecting cognition – intoxication, withdrawal, medical causes (delirium), PTSD □ leading to delay/inability to conduct assessment
 - c. Stigma, views of MH and past negative experience □ reduced engagement
2. **System Factors**
 - a. Bed flow pressure
 - b. Lack of collateral information – brought in by police, nil NOK available
 - c. High workload of staff – limited time to obtain detailed history from patient
 - d. Legal: need for Guardianship Act – resulting in rupture of therapeutic alliance

3. Staff/Environmental Factors

- a. Availability of security staff
- b. Lack of privacy – in ED, high acuity environment, limited single room availability
- c. High stimulus environment – potential risk for increased hypervigilance/aggression
- d. Prejudice, Counter-transference of nursing staff, eCAT clinician

Three days on, Bill has been admitted to the veteran's ward of your hospital. It has been revealed that he lives with his 65-year-old wife Margaret. Margaret reports that Bill suffered immensely from his wartime experiences. Though he rarely speaks of them, she notices that he often mutters about communists in his sleep, waking in a cold sweat. Sometimes, when a door is shut too loudly, he will "jump out of his seat." He is often grumpy and moping about, but Margaret is used to this. He drinks around 6 stubbies of beer each day and has always done so. In the past few months, Bill has seemed out of sorts. He appears to have lost some weight and the other day, she heard him crying in his shed.

4.2 List your differential diagnoses (5 marks)

1. PTSD
2. Alcohol use disorder/withdrawal/delirium
 - a. (Wernicke's encephalopathy, Korsakoff's psychosis)
3. Major depression
4. Adjustment disorder
5. Personality disorder
6. Psychotic illness
7. Organic causes (Dementia, Head injury, Delirium)

4.3 Outline your short and long term management of Bill (12 marks)

Short-term – risk containment, physical health stabilization, short-term psychological support

1. Risk management:
 - a. Consideration of MHA use
 - b. Behavioral interventions:
 - i. Attend to needs, nurse special
 - ii. Individual room, low stimulus environment
 - iii. Avoid challenging or irritable response
2. Biological:
 - a. IV/IM Thiamine 300 mg over 3-5 days, then oral thiamine 100mg TDS for weeks
 - b. IV fluids
 - c. AWS – diazepam (beware of DT's as 3/7)
 - d. Low dose antipsychotics for behavioral agitation/aggression (haloperidol/risperidone)
 - e. Exclude organic causes – bloods, neuroimaging
3. Psychological:
 - a. Relaxation and anxiety management
 - b. Supportive psychotherapy

Long-term - relapse prevention, long-term psychological support, support for carer

1. Risk management:
 - a. Risk of physical deterioration, risk of misadventure

- b. Monitor mental state for suicidal/aggression risk
 - c. Referral community f/u including alcohol and drug service
- 2. Biological:
 - a. Consideration of SSRI if indicated. Nightmares - prazosin
 - b. Consider naltrexone, acamprosate, disulfiram
- 3. Psychological:
 - a. Trauma-informed CBT, EMDR
 - b. Motivational interviewing looking at level of insight and stage of change
- 4. Social: Carer support, veteran services

MEQ 5

You are a part time psychiatrist working on an inpatient unit of a public hospital. Your registrar Jeremy is in his first year of training. You have been increasingly concerned about Jeremy. He appears unable to manage his workload effectively or prioritize. His clinical skills are also very poor and he regularly 'forgets' to do risk assessments or mental state examination, or chart medications when admitting patients. You are aware it is his first rotation and he must be anxious. You have been patient with him and provided extra supervision to help coach him through these tasks. Nonetheless, he does not appear to be improving. He is often late to work, and you have noticed him falling asleep at his desk. He appears very distressed and frazzled when you or nursing staff provide feedback around things which have been missed.

5.1 Describe the reasons which may be contributing to Jeremy's difficulties (8 marks)

1. Individual factors (6 B's - Blues, Booze, Bird/Bloke, Bank, Bros)

- a. Training factors:
 - i. Junior, lacking experience - unable to prioritize demands of new job role
- b. Substance abuse - poor sleep, hangover effects, intoxication or withdrawal
- c. Depression, anxiety, adjustment disorder, personality vulnerabilities
- d. Coping style/resilience
- e. Physical illness - poor nutrition, low iron, hypothyroidism, EBV, OSA

2. Environmental factors

- a. Hostile/bullying and discrimination - distressed at work
- b. Psychodynamic issues with staff and patient – countertransference

3. Systems factors

- a. Poor support from management
- b. Blame culture
- c. Roster issues/fatigue
- d. Communication and competing requirements (clinical vs training requirements)
- e. Poor orientation to roles and responsibilities causing increased anxiety

NB 'Describe' - needs more details about at least 4 domains, they are focusing on 'depth' not on 'breadth'

5.2 Outline your approach to management of this situation (12 marks)

1. Interview with Jeremy

- a. Open discussion with Jeremy
- b. *Who* - with myself and Jeremy +/- other consultant
- c. *Where* - Private place
- d. *What* - Performance issues, elicit underlying cause (burnout vs depression, substance use)
- e. *When* - as early as possible
- f. *How* - empathic, maintaining confidentiality, non-judgmental (natural justice)

2. Mandatory reporting - assess risks to patients or concerns regarding competence vs risk of professional reputation/development

3. Training factors

- a. Involve director of training
- b. Regular supervision and peer support
- c. Targeted learning plan

4. Systems factors

- a. Consideration for fatigue leave, reduce on-calls until progress has been made
- b. 2x weekly supervision until progress has been made
- c. Liaise with clinical director regarding above

5. Extra supports:

- a. Healthy lifestyle – exercise, diet, meditation
- b. Welfare officer, Income support, Doctors helpline
- c. Peer supervision
- d. GP/psychologist/private psychiatrist

MEQ 6

You are the on-call psychiatrist and also work on the inpatient ward. You receive a call from your registrar about Sharon, a 35-year-old woman with chronic schizophrenia who is admitted under you. Sharon has been admitted earlier in the week for the purpose of a medication change. She had been managed with haloperidol 6mg daily for many years in the community but had suffered akathisia in recent times which was causing her great distress. Four days ago, you changed her to quetiapine 1000mg with some effect. Sharon is now unwell with a fever. The registrar went to assess Sharon but found her asleep and thought better of waking her. The registrar was unsure how to proceed.

6.1 Outline your advice to the registrar around assessment (10 marks)

1. Concerns are sufficient to warrant urgent **physical and mental health assessment**
 - a. Full neurological exam: dystonia, Fluctuating LOC – delirium, Hyper-reflexia, Hemiparesis or CN abnormality (intracranial event)
 - b. Autonomic instability
2. Review all **medical investigations** done so far, consider add-on:
 - a. FBC, E/LFT, CK, RFTs, Blood culture, TFTs - NMS, sepsis, delirium screening
 - b. Bedside tests: ECG - QTc prolongation, Urine MCS
 - c. CT/MRI - intracranial pathology
3. **Information gathering**
 - a. Nurses and medical notes for recent concerns or deterioration in behaviour
 - b. Substance use hx - Consider possibility of withdrawal
 - c. Recent other medications:
 - i. Excessive benzo's
 - ii. Last dose of antipsychotic?
 - iii. Antidepressants (increase risk of serotonin syndrome)
4. Offer to come in if registrar is not confident

Your registrar calls you back and sounds anxious. They have now assessed Sharon. They inform you that Sharon is asleep and difficult to rouse. On waking, Sharon appears confused. She is not responding to the registrar's questions but looks at them blankly. The nursing staff believe that is not normal for Sharon. On examination, she remains febrile and hyperreflexic. Her blood pressure is 180/90, heart rate 100, respiratory rate 20.

6.2 Outline your advice to the registrar around immediate management (8 marks)

1. Medical emergency as likely experiencing NMS or serotonin syndrome (Call code blue)
2. Physical monitoring, 2 large cannulas and take bloods (CK, venous gas, renal function)
3. Fluid and IV support – to treat volume depletion and hyperthermia
4. Stop antipsychotics – as this is likely offending agent
5. Organize transfer to medical team for stabilization (needs ICU admission)
6. Administer dantrolene and bromocriptine (if NMS suspected)
7. Liaise with CL team who will need to see patient on ward
8. Notify NOK, NUM and clinical director about critical incident
9. Offer to come in if registrar not confident, debrief with registrar and nursing staff.

Sharon is now better and you have ceased quetiapine. In light of what happened, you are considering commencing her on Clozapine. Sharon's mother Mary is on the ward and would like to know what your plans are for her daughter.

6.3 Discuss your management advice with Mary and Sharon (10 marks)

1. **Psychoeducation** of Mary and Sharon together:
 - a. **Risk-benefit analysis:**
 - i. Risk
 1. Common S.E.: constipation, hypersalivation

2. Rare S.E.: Myocarditis, Agranulocytosis, Metabolic syndrome
- ii. Benefits:
 1. Clozapine safest due to risk of NMS
 2. Best medication considered treatment resistance and chronic SCZ
 3. 60% of Tx resistant SCZ respond to clozapine
 4. Improved cognitive profile
- b. **Practicalities:**
 - i. Registration with CPMS
 - ii. Needs to be initiated in hospital
 - iii. Weekly blood test for first 18 weeks
 - iv. Monthly blood tests in clozapine clinic after this and regular reviews by community team
 - v. 6/12 metabolic monitoring, yearly cardiac monitoring
2. **Psychological:**
 - a. Consider Cognitive remediation therapy
 - b. CBT for psychosis - potential to worsen psychosis
3. **Medicolegal:**
 - a. Capacity and need for MHA
 - b. Guardianship, substitute decision maker
4. **Other:**
 - a. Rehab focus: Consider CCU or rehab-based team – to enhance functional capacity
 - b. Practical support - NGO referral / peer support
 - c. Carer support
 - d. Lifestyle interventions

NB - remember for the 'discuss' questions, you need to go into more detail and discuss the pros and cons of each point

MEQ 7

You work in a CYMHS outpatient clinic and a pediatrician has sent you a referral. 10 y.o boy attends with his mother. Lives at home with his parents and siblings. Pediatrician has diagnosed him with ADHD and Tourette's and prescribed him methylphenidate and clonidine with improvements in behavior at home. However, he now has poor school performance.

Q 7.1 What are the possible reasons for his poor school performance?

1. Factors which negatively affect cognition and concentration

- a. Medications issues
 - i. Inadequate dose of stimulant medication - uncontrolled ADHD
 - ii. Side effects of medications, e.g. excessive sedation secondary to clonidine
 - iii. Excessive stimulant medication resulting in increased hyperactivity, sleep disturbance and poor nutrition
- b. Untreated co-morbid mental illness
 - i. Anxiety
 - ii. Depression
 - iii. ASD (more vulnerable to medications and acute stressors)
 - iv. Substance abuse
- c. Incorrect diagnosis/missed organic cause – inconsistent response to medication, worsening symptoms at school
- d. Trauma e.g. physical, emotional, sexual, neglect which may lead to dissociation and poor school performance

2. Lack of factors that promote cognition and concentration

- a. Insufficient psychological interventions e.g. Behavioral analysis and cognitive therapy to address school performance
- b. Limited parental support due to financial strain/relationship breakdown (exacerbated by specialist costs etc.)
- c. Inadequate staffing, poor teacher skill
- d. Lack of suitable environment to complete schoolwork (e.g. noisy siblings)
- e. Lack of psychoeducation for teachers

3. School refusal

- a. Bullying - secondary to stigma around mental illness and uncontrolled Tourette's
- b. Stigma causing prejudice against child
- c. Anxiety

Mother reports that she has read in a magazine that stimulants cause growth retardation and drug addicts

Q 7.2 How do you respond to mothers concerns about medication

1. Acknowledge and explore concerns, providing validation of difficult situation to maintain therapeutic alliance
2. Provide psychoeducation to increase understanding of treatment of ADHD:
 - a. Risk-benefit analysis
 - i. Risk of stimulant abuse, however risk mitigated by adequate symptom control

- ii. Risk of growth restriction secondary to appetite suppression with stimulants, however this is mitigated with variations in dosing regimes (e.g. drug holidays)

b. Management strategies

- i. Consideration for long-acting stimulants which minimizes risk of stimulant misuse
- ii. Multimodal approach with psychosocial and pharmacological interventions to achieve symptom control
 - 1. Psychotherapeutic interventions to boost protective factors (e.g. CBT, developing healthy coping strategies)
 - 2. Lifestyle interventions – exercise, diet, healthy relationships
 - 3. Social skills training
 - 4. Parent training
- 3. Organize regular follow up for patient and mother for monitoring of response to treatment, monitoring of growth parameters, and address outstanding concerns
- 4. Liaison with GP/pediatrician to ensure adequate medical input and monitoring

You continue to prescribe him his medication and his school performance improves. However his mother is concerned about ongoing tics

Q 7.3 Outline your management of his Tourettes

1. Multimodal assessment to guide management

- a. Speech and OT Assessment – to assess type and level of impairment
 - i. Simple vs complex tics, motor/vocal
- b. Effect of current treatments (methylphenidate and clonidine) on tic severity
- c. Check for comorbidities e.g ODD, anxiety (may affect response to treatment)
- d. (NB Dx: At least 2 motor and at least 1 vocal - dx of Tourette's)
- e. Collateral information: school, parents, GP
- f. Discussion with child re: distressing symptoms and effects on his mental health

2. Discussion of evidence-based strategies from a biopsychosocial perspective:

- a. **Psychological –**
 - i. Behavioural reversal training
 1. Identify precursors to tic onset
 2. Perform voluntary movement that is incompatible with performance of tic
 - ii. Psychologist – Cognitive Behavioural Intervention for Tics (CBIT)
- b. **Biological interventions:**
 - i. Discuss risks vs benefits of pharmacological options
 - ii. Clonidine - alpha adrenergic antagonist, guanfacine
 - iii. Neuroleptic (low dose haloperidol or risperidone) or dopamine depleting agent - tetrabenazine
 - iv. Consider altering dose of stimulant as can worsen tics - consider atomoxetine
- c. **Social Interventions**
 - i. School
 1. Liaise with school counsellor - extra support
 2. Psychoeducation to de-stigmatize tourette's and ADHD & reduce bullying
 - ii. Parents / family meeting
 1. Psychoeducation re: treatment and prognosis for illness (many resolve by adulthood)
 2. Discuss with siblings at developmentally appropriate level

3. Long-term

- a. Arrange regular follow-up to monitor response to medication & symptom progression
- b. Link with CM, GP and paediatrician

MEQ 8

6 year old boy with encopresis, mother is on single parent support. Referred by his paediatrician who has been treating him with laxatives and star chart for six months to no effect. He has been reviewed by a surgeon to exclude medical causes.

8.1 Outline your assessment (6 marks)

1. Recent history to clarify diagnosis:

- Precipitating and perpetuating factors – triggers, stressors (bullying)
- Primary vs secondary encopresis
- Duration, onset of encopresis
- Current functioning – living situation, attachment style, school performance, peer relationships
- Treatment strategies used, effects, compliance
- Comorbid symptoms -urinary issues, pain

2. Past history to elicit predisposing factors for encopresis:

- Medical history
- Developmental history – milestones, developmental delay
- Psych Hx - anxiety do (separation), conduct issues
- Family history – carer's mental health, previous psychiatric diagnoses, current coping, dynamic factors
- Prejudicial factors such as trauma, neglect etc, previous child safety involvement

3. Observation of the child/Mental State Exam to elicit signs of mental illness/distress:

- Parent-child interaction and attachment style (focus on behaviour and affect)

4. Risk assessment with focus on abuse/neglect as cause of encopresis

- Suspicious lesions/bruising, unexplained past injuries
- Maternal factors - depression, suicidal, poor functioning

5. Quality of supports to inform management

The boy's grades are normal, and he has friends, though the kids sometimes make fun of him for being smelly.

8.2 Describe your management

Biopsychosocial and multidisciplinary approach to management:

1. **Biological interventions**
 - a. Manage organic causes of incontinence – e.g. laxatives for constipation
 - b. Encourage fibre in diet, regular fluids to promote healthy toileting behaviours
2. **Psychological interventions:**
 - a. Behavioural intervention to promote normal bowel habits – behavioural analysis, ABC, bowel training
 - b. Psychoeducation – for parents, school
3. **Social interventions:**
 - a. Practical supports at school – extra set of clothes, encourage hygiene
 - b. Carer support

Liaison for multidisciplinary approach:

1. Paediatric specialist, GP /paediatric input
2. OT or child psychologist
3. Discuss case with senior colleague/child psychiatrist

His mother informs you that the father is out of the picture. She feels her son is 'violent' like his father. She is also getting tired of "rewarding him, when he doesn't deserve it"

8.3 Describe your management

1. **Obtain more information to clarify specific concerns:**
 - Circumstances of separation and impact on children
 - Family history of mental health illness, especially parents
 - History of domestic violence in household – towards her and the children
 - Current supports, mother's coping style
2. **Risk assessment**
 - Risk of violence of son towards mother
 - Risk of abuse by mother – emotional abuse (incl projecting past difficulties onto children), neglect, homicidal thoughts, maternal substance use
 - Risk of burn out, maternal suicidal risk
3. **Multimodal approach to maximise scope of interventions**
 - Psychoeducation to improve understanding - children's behaviour are often an expression of unmet needs, emotional/ social difficulties, or underlying developmental difficulties
 - Support for mother:
 - Parenting skills - Circle of Security, PPP - emphasizing importance of boundary setting and consistency
 - Seek own psychological support e.g. via GP and Mental Health Care Plan
 - Consider referral to Family and Child Connect

Sam has not seen his father for some time. His mother has depression and is on antidepressants. His mother said that she is tired of bribing him to behave. When you see Sam, you notice that he has a bruise on his arm.

8.4 (alternate) Outline your immediate management plan.

- **Approach / obtain information**
 - Sensitive approach with consideration for possible child abuse, but avoiding accusations
 - Ideally need to review Sam separately from mother
 - Enquire about Sam's wellbeing and assess interaction between mother and Sam
 - Further delineate mothers concerns about behaviour, current management strategies

- Risk screen any harm to Sam and if he has any violent behaviours
- Assessment of Sam
 - Use non-leading line of enquiry to avoid undue influence as Sam may be highly suggestible in a vulnerable state (i.e. a child)
 - Ask Sam if he has been hurt by his mother or anyone (e.g. mother's new partner)
 - Physical examination because he has bruises, which may be a sign of physical abuse. There may be other injuries which may need urgent attention in the emergency department for full assessment.
- Support / treatment for mother
 - Assess mother's current mental state
 - Assess current supports - consideration of adult mental health service referral to optimize her management, as this will increase her ability to meet Sam's needs
 - Collateral/liaison from GP/treating MH team in regards to maternal mental health and functioning to assess risks and guide further management
 - Is there any relatives /extended family who could provide support for her
 - Psychoeducation and consideration for referral to parenting program
- Child safety considerations
 - Discuss case with hospital Child Protection Liaison Officer
 - Based on assessment consider if child safety involvement recommended
 - Could involve social worker and hospital child safety if indicated
- Follow up / future considerations
 - Set up appropriate follow up appointments and involvement of her GP, DOCS and involve family

MEQ 9(

Olivia is a 29-year-old, married mother of three children aged twelve months to eight years. She experienced significant sexual abuse in her childhood. Olivia has just been discharged to the local community mental health team at which you are the consultant, following a three-month admission to a nearby acute psychiatric inpatient unit.

Prior to admission Olivia became depressed after hearing that her brother had been found guilty of sexually assaulting his stepdaughter. She developed flashbacks and nightmares of her own experiences of abuse, and had frequent panic attacks. She was found by her oldest child with a cord around her neck after trying to hang herself, and admitted to hospital.

Her hospital admission was characterized by recurrent episodes of self-harm including lacerating her arms and tying cords around her neck. She was treated with sodium valproate, paroxetine and diazepam. Olivia was eventually discharged on these medications still symptomatic.

Question 9.1 Discuss the immediate and longer-term psychiatric interventions you would suggest for Olivia's management (previous sexual abuse). Please give reasons for your choices. (7 marks)

Immediate

1. Risk and safety

- Assess symptoms and risk to guide inpatient vs outpatient treatment
 - i. any ongoing thoughts or acts of self harm or suicide
 - ii. risks to others – safety of children, exposure to maternal self harm, level of support from husband, is a referral to child safety services warranted
 - iii. Access to means including medication, firearms, and ropes
- If not for readmission, for assertive case management
- Liaise with inpatient team re: details of admission and rationale of discharge

2. Biological

- Review current medications and compliance
 - i. Review diazepam due risk of dependence and tolerance, aim to wean as clinically appropriate
 - ii. Check sodium valproate levels to monitor adherence. Exclude pregnancy and ensure contraception to avoid risk of teratogenicity.
 - iii. Monitor response to paroxetine and titrate accordingly
 - iv. Consider clonidine, prazosin for nightmares
- Exclude active substance use

3. Psychological

- Strategies to manage hyperarousal including grounding techniques, relaxation, mindfulness
- Create a safety plan and make aware of crisis contacts

Longer term

4. Trauma specific psychological intervention

- Trauma focused CBT or DBT when Olivia is more stable and able to address trauma without significantly elevating risks
- Consideration of EMDR
- Review child mental health and consider psychology referral

5. Social/lifestyle

- Regulate sleep/diet/exercise
- Minimise stressors and relationship conflicts
- Support for parenting duties

- Vocational support - Centrelink support/medical certificates

6. Biological

- Continuing to observe for response to treatment and review medication regimen
- Monitor response
- Referral to drug and alcohol supports if relevant
- Review diazepam and wean as appropriate

Question 9.2 Discuss the interventions you would use to support Olivia's family. Please give reasons for your choices. (8 marks)

Engagement with Olivia to maintain therapeutic alliance and respect privacy

- Consent to engage with family members
- Discuss what is important for Olivia regarding confidentiality (parameters of what she is happy to disclose)
- Offer family meeting and support
- Psychoeducation to Olivia about potential impact on family from witnessing self harm

Support for 8-year old child because he witnessed self-strangulation and is at risk for trauma

- Offer assessment by child and adolescent service for symptoms of trauma/ptsd
- Liaising with school to assess if school functioning impacted and collateral history
- Consider specific services if available - eg Children of Parents with Mental Illness (COPMI)

Support for the other children because they may not have needs met in the context of maternal mental illness/absence of care

- Age appropriate interventions otherwise
- For 12 month consider attachment needs including extended family who could be involved in care. Consider if breast-feeding still occurring.

Support for husband because he is primary carer for Olivia and children and may be overwhelmed/distressed

- Psychoeducation about PTSD, risks, and strategies to manage acute symptoms
- Safety planning - threshold for contacting crisis support/emergency services
- Offer support for his own mental health concerns
- Consider referral to consumer care officer, additional NGO supports
- Explore financial issues and possible entitlement for carer's allowance

Support for extended family and wide support network as they may be involved with care of children, and would require further support

- Family safety planning including where children can stay in case of crisis admission
- Referral to support agency for family (e.g. family and child connect)

Olivia has just found out that she is pregnant again as a result of having unprotected sex with her husband when on weekend leave from hospital. She and her husband are concerned about the effects of her psychotropic medication on the baby.

Question 9.3 Outline the information and options you would discuss with Olivia and her husband. Please provide adequate detail. (10 marks)

****Perinatal Learnit****

Confirm pregnancy to determine gestational age to guide further prescribing

Understand patient's attitude towards pregnancy and concerns:

- Planned/unplanned, wanted/unwanted
- Do they intend to continue or terminate pregnancy

Specific psychoeducation to guide informed decision making for Olivia

1. Discuss limitations of evidence-based management due to limitation of research with pregnancy
2. Risks, benefits and consequences of receiving medications
 - a. Sodium valproate has 5-17% risk of congenital malformations including increased risk of neural tube defects
 - i. Baseline risk for general populations 3-5%
 - b. Paroxetine also carries increased risk in first trimester for increased congenital and cardiovascular malformations, and increased neonatal respiratory distress due to discontinuation/withdrawals
 - c. Diazepam carries a risk of neonatal withdrawal symptoms
3. Risks, benefits and consequences of not taking medications
 - a. Explain the risk of untreated mental illness including parental stress effects, poor attachment
 - b. Increased risk of developing post-partum depression given past history of depression (independent of medications)

Depending on Olivia's decision, these could be risk mitigation interventions

- High dose folate (5mg/day)
- Risk/benefits of ceasing/changing mood stabilizer and antidepressants
- Consider inpatient re-admission for medication management
- Provide psychoeducation about warning signs of relapse

Perinatal planning to ensure safety due to complex mental health needs:

- Discuss pre- peri- and post pregnancy care
- May require planned admission closer to delivery
- Development of birth management plan with CL-psych/perinatal MH input
- Explore wish to breast feed as this will guide medication management

Referral to services for all-encompassing care:

- Referral to obstetrician and/or high risk pregnancy clinic ideally with MDT perinatal mental health service input
- Ongoing liaison with GP for antenatal care
- If Olivia wants to terminate pregnancy, to refer to appropriate services

Support

- Provide written information
- Allow time to consider decision
- Schedule follow up appointment
- Emphasize to husband importance of support to Olivia

MEQ 10

50yo Emil Refugee man, has been at an asylum 8 years where he witnessed atrocities & trauma. Married with 2 teenage children. Treated previously with multiple antidepressants but neither successful. He currently presents with Insomnia, low motivation. Unemployed & restricting himself to the house. He has difficulty speaking English & has limited supports.

10.1 What collateral & from whom would be important to gather before you see him?

- Clarify **visa status and application** - recent rejections or alternative settlement option. If rejection would see Emil returned to dangerous situation
- Wife and family to understand baseline functional capacity and recent precipitants
 - o Current functional status – clarify employment, housing, social functioning
 - o Review of symptoms of mental illness specifically screening for depression, trauma symptoms, suicidality, OCD behaviours, and substance use, neuroveg, anxiety symptoms – social anxiety, agoraphobia
 - o Any specific concerns from wife or children related to their safety or otherwise
 - o Background information on cultural factors, circumstances for refugee seeking, premorbid personality, recent changes, psychiatric history, family psychiatric history

GP/referrer – to elicit pre-existing psychiatric history

- Medical history – nutritional deficiencies leading to symptoms of depressed mood, thyroid dysfunction
- Psychiatric history – previous diagnosis of psychosis, PTSD, anxiety symptoms – agoraphobia, depression
- Medications – which antidepressants, course, compliance, responsiveness
- Recent change and reason for referral
- Level of English, whether interpreter required

Asylum organisation

- History of past diagnoses and treatment - both in detention and prior
- Observation of behaviour in asylum
- Culturally relevant information - religious, conceptualisation of mental illness

Public mental health database

- History of past diagnoses, treatment, contacts
- Review alerts and past risk information
- Past admissions

10.2 List your differential diagnoses at this point

Likely a co-morbid mix of the below;

- MDD
- Adjustment disorder with depressed or anxious mood
- PTSD
- Agoraphobia or social anxiety or other anxiety disorder
- Substance use disorder – particularly alcohol
- Psychotic disorder
- No psychiatric diagnosis – possible issues with acculturation, coping, demoralization
- Medical cause leading to symptoms – hypothyroidism, anaemia, deficient b12/folate/vit D, neuropsychiatric Sx due to infective cause – eg. Syphilis, HIV

10.3 How would you manage this patient

Approach

- Interview with interpreter and cultural liaison/transcultural MH
- Consider interviewing Emil alone and with family

Consider appropriate setting

- Inpatient vs outpatient
- Consider home visit with transcultural MH
- Assessment of risk
- Voluntary vs involuntary

Treat psychiatric condition

- Formulation that includes patients explanatory model of illness with cultural liaison
- Consider medications for mood/anxiety/psychosis
- Address substance use if present
- Psychological interventions such as for PTSD

Support and follow up

- Psychoeducation and support for family
- Ongoing assertive follow up
- GP follow up – for medical health

Referral

- To refugee support service - additional community groups to assist in integration into local community
- Consider wellbeing of other family members - in particular exploration of child welfare and MH given adverse conditions
- Referral to transcultural MH if available

Meq 11 (Aug 2016 RANZCP)

Omar, regular cannabis & other drug user, 2 CTOs in the last 5yrs. On risperidone depot & lithium, parents are doctors & have been prescribing & checking his levels. He has returned to live with his parents over the last 2 months after living away for 18mth under the condition that he remains compliant with medications.

11.1 Questions you wish to ask to decide on management?

Seek collateral from parents, GP and prior specialist services

- Risk assessment: determine nature of illness through past presentations and admissions, risks when unwell both manic and depressed, longitudinal capacity, personality and coping style, function when well and unwell
- Compliance with treatments historically - use of a depot or webster packs
- CTO information - is Omar voluntary or involuntary, past ECT
- Medications information - what has been tried and response. Metabolic concerns or EPSE
- Mental state information - current and fluctuations with substance use
- Risk information - to self, others, vulnerability financially and to relationships
- Recent substance use information - type, route, impact on mental state. Treatment for these?
- Medical co-morbidities - frequency of blood monitoring and history of renal or thyroid issues
- Level of independence and ability to seek help
- Family issues/dynamics - reasons for relocation and engagement of supports. Attitudes to substance use

11.2 Omar Wants to know the risks of the antipsychotic & mood stabiliser he's on?

- Lithium
 - Small therapeutic window (small range of therapeutic response without causing significant adverse effects) – hence requires regular blood monitoring
 - Risks of toxicity if level too high and side effects
 - Lethargy, insipidus (increased urination), dehydration, tremor, headaches, GI changes (diarrhoea/nausea), memory impairment/delirium, metallic taste, coma, death
 - Sedation, hair loss, weight gain
 - Thyroid dysfunction - usually hypothyroidism
 - Kidney dysfunction in long term use
 - Some evidence for hypoparathyroidism
 - Risks of drug-drug interactions (eg. NSAIDS, diuretics)
- Risperidone
 - Sedation
 - Metabolic syndrome such as weight gain, obesity, diabetes, dyslipidemia (high cholesterol levels) and other cardiovascular risk factors
 - Extrapyramidal side effects - parkinsonism, akathisia, dyskinesia (including tardive), dystonia (involuntary tremors, restlessness)
 - Hyperprolactinaemia effects - sexual dysfunction (erectile dysfunction, reduced sperm production), menstrual changes, breast tenderness and galactorrhoea (milk production)
 - Postural hypotension (positional changes in blood pressure which may lead to dizziness and falls)
 - Seizures

Dad states he wishes for his son to be reviewed only by the consultant & although the son seems to get along well with the registrar, he wants his son to be reviewed by the consultant.

11.3 How would you respond to this request?

- Provide father opportunity to express his concerns sensitively
 - Allow father to identify concerns that they have with the registrar
- Ethical considerations
 - Autonomy: Consider Omar's preference with care
 - Justice of equal provision of care
 - Beneficence:
 - Importance of maintaining therapeutic alliance with Omar and registrar
- Psychoeducation about model of service
 - Case management approach
 - consultant is involved in the care and MDT case reviews occur regularly
- Options to provide feedback and complaints

11.4 Dad wants IPST Interpersonal Social Rhythm Therapy – describe what it is & how it might benefit this pt?

- IPSRT considers the role that biological and social rhythms play in managing bipolar disorder fluctuations. Role as an adjuvant therapy to medication
- Can be offered individually and in a group setting - commences with review of history and formulation before progressing to more targeted skills. Typically offered in four phases
- Biological rhythms considerations - impacts of sleep cycle, medication stability, substance use on alterations to mood
- Social rhythm and interpersonal considerations - identifying stressful life events and relationships that impact on mood stability and how to manage these
- Highlight importance of family/cohabitants involvement
- Focus is on building a sustainable structure to daily life that best minimises risks for relapse
- Provide resources - website developed by those who developed www.IPSRT.com

11.5 Dad has prescribed and given the depot again. How would you handle this situation?

- Patient safety – ensure patient has not been given double dosing
- Arrange family meeting to discuss rationale for this and any observed deterioration in mental state prompting this decision
- Discuss issue with clinical director, medicolegal representative with regard to medicolegal responsibilities i.e. reporting
- Outline concerns and emphasise the importance of boundaries, referencing clinical guidelines from AMA and highlighting lack of objectivity and risks this poses
- Emphasise medication management to be reviewed and administered by treating team moving forward with open communication with family/supports in planning this

MEQ 12 (RANZCP Feb 2017)

50 year-old woman presents to ED. Has her 15 year old daughter with her. Reg calls she has acute pancreatitis and thinks she is in alcohol withdrawal and hallucinating and has given diazepam.

Q12.1 what's the information you want from the reg (14 marks):

1. Confirm withdrawal state with AWS to confirm the clinical picture
 - a. Tremor, diaphoresis, tachycardia and hypertension
2. Clarify nature of hallucinations, cognition and sensorium to assess presence of delirium
 - a. Fluctuating consciousness, impaired attention, rapid onset
3. Collateral - from daughter and additional adult sources to identify any recent risks and explore aetiology of illness
 - a. Timing, onset, duration of behaviour
 - b. Concerns of risks
4. Consider delirium tremens - history taking to rule in or out diagnosis
 - a. Hx and duration of alcohol use
 - b. Time since last drink, Current BAL
 - c. Diazepam, thiamine
5. Need to exclude other organic causes/delirium of presentation to guide correct treatment **NB important as premature diagnosis can lead to inappropriate use of sedatives and delay accurate diagnosis.**
 - a. Head trauma - ICH or SDH, risk of falls secondary to alcohol intoxication
 - b. Drug overdose/intoxication - increased likelihood of other illicit drug use
 - c. Hepatic failure and GI bleed – sequelae of alcohol dependence
 - d. Hypoglycaemia - check BSL – restricted food intake due to alcohol use
 - e. Sepsis - fever, SIRS response? – from pancreatitis
 - f. Pancreatitis and acute pain
6. Assessments and investigations with aim to manage preventable complications
 - a. Full neurological examination – e.g. oculoplegia, ataxia, focal neurological signs – consider Wernicke's, head injury
 - b. Sequelae of chronic alcoholic liver disease – jaundice, caput medusae, spider naevi, elevated JVP, hepatomegaly
 - c. Investigations: FBC (anemia, infection), glucose, lipase/amylase (pancreatitis), LFT (liver disease), CT head (head injury), urine MCS (infection), renal function (dehydration secondary to pancreatitis)
7. Review current treatment in order to determine if adequate and how to manage any gaps
 - a. Thiamine IV 300mg before receiving glucose, response to diazepam
 - b. Environmental cares for delirium: nursing special, single room, low stimulus
 - c. Current management for acute pancreatitis
8. Current risks - need review in order for management to attempt to mitigate these
 - a. Late presentation delirium tremens carries significant risk mortality
 - b. Risks to self - harm due to misadventure/agitation, ?suicide attempt
 - c. Risks to others from aggression - level of agitation
 - d. Risk to dependents - Care arrangements for 15 year old daughter

The registrar calls back reporting that the patient now has developed double vision to lateral gaze and reduced consciousness

Q12.2 Outline your differentials and give an immediate management plan (8 marks)

Differentials

1. Wernicke's encephalopathy (thiamine cofactor for energy metabolism - reduced in alcoholics due to poor absorption, inadequate intake and reduced hepatic storage)
 - a. Wernicke's triad: encephalopathy, oculomotor dysfunction, gait ataxia
2. Alcohol withdrawal delirium (DT in most severe cases) or alcoholic hallucinosis:
 - a. Usually occurs 72-96 hours after last drink

- b. Focused hx looking for prior withdrawal, chronic illness (e.g. cirrhosis, thrombocytopenia), concurrent illness (e.g. resp, cardiac)
- 3. Delirium (due to medical compromise)
 - a. Overdose - anticholinergic
 - b. Respiratory, urinary or infective causes (meningitis)
 - c. Signs of acute abdomen - peritonism
 - d. Ix: increased lipase and amylase, CT showing inflammation
- 4. Head injury or intracerebral cause (infarction or haemorrhage)
 - a. CT or MRI to rule out structural lesion
- 5. Primary psychotic illness – unlikely to consider until medical causes ruled out
 - a. Look for hx of psychosis & psychotropic meds - compare with current presentation
- 6. Drug intoxication (e.g. LSD, PCP)

Immediate management plan:

- 1. Legal issues around treatment and consent:
 - a. If lacks capacity, treatment to be considered under guardianship (emergency or with consent of substitute decision maker)
 - b. Consider use of mental health act legislation if signs of primary mental illness
- 2. Medical interventions:
 - a. Given possible Wernicke's give thiamine 300mg IV TDS until clinical improvement
 - b. Commence CIWA (Clinical Institute Withdrawal Assessment for Alcohol Scale)
 - c. Benzodiazepines for alcohol withdrawal: PRN dosing as per AWS until a maximum of 100mg, with staff to contact CL team if more required
 - d. Beware giving benzo's for other delirium causes as likely to worsen confusion
 - e. IV fluids and supplementary vitamins
 - f. Consider low dose anti-psychotic for agitation - small frequent doses, beware of large doses as pt medically compromised
- 3. General delirium management:
 - a. Supportive care, low stimulus environment, single room, 1:1 nurse, presence of family member to calm pt, regular re-orientation prompts
 - b. Medical emergency - treatment under medical team with input from CL team
- 4. Provisions of care for the child given hospitalisation needed
 - a. Contact other family, secondary guardian etc
 - b.** Liaising with CPLO, social worker if necessary

MEQ 13 (RANZCP Feb 2017)

You are old age psychiatrist. Mrs Cross, 74yo, retirement home. You did home visit and found her to be acting bizarrely, not sleeping, Irritable and has cognitive deficits across many domains. Presents with reduced MMSE 21/30, clock drawing abnormal, Frontal lobe deficits and confusion. On lithium, L-Dopa, Fluoxetine and atenolol.

Q13.1 List your differentials (10 marks)

1. Delirium: confusion, sleep wake cycle disruption, cognitive deficits, disorganised behaviour.
 - a. Gastrointestinal, Renal, Infection, Vascular, Electrolyte
 - b. Medication/polypharmacy – AEC score is <3 so less likely to be the cause
2. Dementia related (cognitive deficit, MMSE of 21/30, abnormal clock drawing)
 - a. BPSD - Progressing dementia severity (vascular, AD, FTD) (she is on atenolol which may be prescribed for HTN)
3. Iatrogenic causes of presentation:
 - a. SSRI causing SIADH hyponatremia or from abrupt withdrawal – may be dehydrated
 - b. Li-either due to thyroid abnormalities or kidney impairment or due to toxicity can cause delirium – may be secondary to dehydration
 - c. L-Dopa-could precipitate psychosis which could present as disorganised behaviour, not sleeping, been irritable.
 - d. B-blockers associated with depression which could be masked as pseudodementia
 - e. Polypharmacy – additive effects causing cognitive side effects and subsequent symptoms of irritability and bizarre behaviour
4. Organic due to age and neurological/cognitive deficits that indicate brain pathology:
 - a. Stroke/CVA (as she is on atenolol which may indicate hypertension)
 - b. Lewy Body Dementia
 - c. Parkinson's disease (also is on L-dopa)
5. Psychiatric Causes
 - a. Late onset psychosis- present with disorganised behaviour, paranoia and voices.
 - b. Relapse of BPAD (on lithium and fluoxetine used in the management of BPAD)
 - c. Agitated depression/treatment resistant depression – as she is on fluoxetine and lithium (which can be used to augment ADT therapy). Atenolol can also exacerbate depression
6. Substance related -alcohol or benzo withdrawal/intoxication
7. Complicating patient factors: pain, constipation, dehydration, sensory impairment could present with behavioural disturbances.
8. Complicating staff factors: poor staff communication, poorly trained staff, more unwell patients, elder abuse

Q13.2 What medication advice would you give in this situation and what are the prescribing issues for this patient? (8 marks)

Medication Advice:

- Medication review to identify risks of polypharmacy and optimise treatment
- Review indication for all medications and rationalise as indicated
 - Is atenolol prescribed for HTN or cardiac issue or anxiety?
 - Is lithium prescribed for BPAD or to augment therapy in depression?
 - Involve community pharmacist to assist with deprescribing advice
- Avoid use of benzodiazepines to manage behavior (especially avoid if delirium is confirmed). Also note risk of falls with benzos
- Recommend quetiapine given less associated exacerbation of Parkinson symptoms
 - Only use if at high risk of harm to self or others

- Start low, go slow
- Consider reduction/cessation of fluoxetine if evidence of hyponatraemia

Prescribing Issues:

- General considerations of medications:
 - Reduced Li clearance with age is a toxicity risk – check level
 - If changing beta blocker to ACEi – note that lithium with ACEi could increase levels - toxicity risk
 - SIADH with fluoxetine (consider change to mirtazapine) - risk of delirium, falls, cognitive impairment
 - L-dopa - potential to worsen confusion and psychosis (may need to liaise with neurologist)
 - Atenolol and postural drop/risk of falls
 - Antipsychotics - beware of EPSE especially as these will be exacerbated in LBD or Parkinson's
 - If administering pain relief – note the risk of renal impairment with combination of lithium and NSAIDs
- Assess capacity to make decisions on medications and provide informed consent as may require consent from NOK/guardian if unable to demonstrate capacity
- Improve compliance – e.g. Webster Pack, simplify regimen to ensure mistakes are avoided

Her son contacts you and wants to make a will, citing that she is frail.

Q13.3 How would you assess her testamentary capacity as son wants her to make a will?

1. Psychoeducation to son:
 - a. Explain the difficulty of assessing testamentary capacity during acute period of unwellness
 - b. Pt's capacity likely to be affected but reversible
2. General principles of capacity - decision and time specific
 - a. Understanding of relevant information pertaining to the decision
 - b. Appreciation - can she explain how the information applies to oneself/ insight
 - c. Reasoning - ability to compare information and infer consequences (of her choice and alternatives)
 - d. Expressing a choice - can she state their preference
 - e. Needs to be free from undue influence or coercion
3. Aspects that Mrs Cross needs to understand are:
 - a. The extent of her assets
 - b. How she wishes to divide her assets
 - c. Justification for why she wishes to divide her assets in the manner chosen
 - d. Has she considered the consequences of her decision and alternatives: e.g. how various family members may feel with the division
4. Collateral information from GP, family, or nursing home to assess for consistency of decision making
5. Advise patient to involve her lawyer
6. Document discussion clearly in pt's progress notes

She wishes to make an enduring power of attorney for property affairs

Q13.4 What information does she need to have to allow this to be arranged

1. Legal issues
 - a. Legal document that can be revoked at any time while she has capacity
 - b. Solicitor/JP must be involved – it is a legal document

2. Capacity/consent issues

- a. Determine motivation behind making EPOA. Freedom from undue influence is an important point.
- b. Determine choice and reason of substitute decision maker. Explain that substitute decision makers have complete authority to do whatever they wish, however she may set limits on this.
- c. EPOA will only come to effect once she loses capacity
- d. Assess if she able to understand this information, see how it applies to her, understand the consequences and express her choice
- e. Original document should be placed in safe place (e.g. bank), however copy should be accessible to her and her carers/NOK
- f. Encourage her to consider appointing EPOA for healthcare decisions (as this is specific to property only), otherwise NOK/Public Guardian would be EPOA when she loses capacity.

****for Healthcare purposes – consider role of AHD.**

MEQ14(RANZCP Feb 2018)

A 14 year old girl presents with her mother, referred by the GP for assessment and management.

She has chronic fatigue syndrome, diagnosed by the GP. She also stays up to 3AM and then sleeps through until midday. She is homeschooled and does not socialize with peers but says she would do this if they could do it at night.

She has been thoroughly investigated by the GP, with no medical abnormalities.

14.1 Outline how you would approach the assessment of this girl?

History to be obtained from patient and mother independently and also opportunity of observed interaction. Consent from both parties with consideration of competence

Review GP collateral and completed investigations - need to ensure adequacy of work-up and exclude organic causes

- FBC, CHEM20, EBV, TFTS, IS, VITD, B12, FOL, hormone levels
- BP/HR/postural values. BMI
- Other medical or psychiatric comorbidities

Full review of sleep to assist in formulation of presenting complaint and extent to which this contributes to dysfunction

- Total quantity, whether wakes feeling rested
- Initiation vs maintenance difficulties
- Normal night time routine - caffeine, electronic device, social media, gaming
- Pattern of daytime fatigue, Day time naps
- Interventions to date and effectiveness

Nature of chronic fatigue ensure meeting criteria for diagnosis and understand trajectory of illness with associated precipitating and perpetuating factors

- Onset, duration - social or personal circumstances at onset,
- Exacerbating/perpetuating factors (relation to sleep, school, parental relationships)
- Impact on function - social interactions vs school attendance, friendships, home chores
- Associated symptoms such as muscle or joint pain, headaches, anorexia

Full psychiatric screen to exclude co-morbidity or assess impact on presenting complaint

- Mood symptoms - diurnal variation, relation to sleep, anhedonia
- Neurovegetative screen - appetite, weight changes, concentration
- Anxiety symptoms - social, generalised and separation

- Clues for somatisation/functional neurological disorder
- Psychotic screen
- Substance screen
- Risk screen - past and current behaviours, SI
- Personality and coping style
- Dynamic factors - adoption of the sick role, primary/secondary gain

Social/Developmental Hx primarily to add to formulation on the development of symptoms and reasons for treatment resistance

- Attachment style, history of separation anxiety
- Enmeshed relationship
- Parent ability to set boundaries, consistency between parents
- Parental dynamic and substance use issues
- Parental mental health issues, coping and resilience
- Effect of sibling relationships - rivalry, sickness,
- Trauma screen - physical/emotional//sexual/neglect
- Peer relationships
- Reason for homeschooling - ?caused by school refusal, bullying issues
- Family stressors -financial stress, losses, sickness
- Extra curricular activities

Mental state - consistent with history and to provide evidence for or against diagnostic hypothesis

- Evidence of low mood or restricted affect
- Level of engagement in review and rapport
- Anxious distress or hypervigilance

Examination

- Evidence of hypothyroid
- Malnutrition

Collateral to clarify aspects of history and gain 3rd party opinion of factors leading to presentation

- Mother / father/ older sibling if possible - looking for differences in perspectives
- Previous school if consent obtained - report cards etc

14.2 Outline your the different psychiatric issues that could be contributing to this presentation?

- Secondary insomnia - poor sleep hygiene/behaviours leading to persistent fatigue
- Anxiety spectrum disorder - generalised, social, separation, school refusal leading to abnormal sleep patterns and avoidance
- Depressive illness - MDD, adjustment disorder, atypical depression manifest by poor energy and social withdrawal
- Substance use disorder - alcohol, cannabis, prescription meds impairing sleep wake cycle or exacerbating underlying primary psychiatric illness
- Dynamic considerations - enmeshed relationship/long standing attachment style, bullying, trauma that foster avoidance and perpetuate loss of function
- Somatisation, functional neurological disorder, sick role considerations
- Parental stressors and mental illness
- Internet gaming disorder – due to staying up late, social isolation, poor sleep hygiene

The mother also reports concern over the patient's lack of sleep which is affecting her own sleep and making her stressed. She has been trialled on multiple hypnotics in the past with minimal success. She is asking you to be prescribed a new medication.

14.3 Outline how you would approach this situation?

- Acknowledging mother's difficulties / validate stress in order to build alliance
- Role distinction - emphasise the need for separate treating doctor for mother as this is an ethical boundary
- Provide psychoeducation to mother about insomnia in general and that medication for this is usually used as a last resort. Explain that there is a risk issue with prescribing sleeping medications as she may be too sedated to manage the daughter's needs
- Discuss respite options – if mother is not coping, there may be a safety issue (assuming that there are no other carers). Consider whether Child Safety/Family and Child Connect may be able to assist in the home
- Consider involvement of mother's GP for MHCP /psychology/counselling/psychiatrist referral to enable better support or treatment
- Brief psychoeducation on role of hypnotics in potentially worsening on anxiety and stress management as justification for suggestion that these not be provided

MEQ 15 (RANZCP Feb 2018)

You are an inpatient consultant in a busy city public hospital. You have admitted Mary, a 49 year old woman with a history of depression. She was diagnosed in her 20's and has had two long admissions to hospital in the past. She also has a history of a manic episode in her 20's. She has a FHx of suicide with two uncles having committed suicide in their 40's.

She has stopped eating and is rapidly losing weight. She is uncommunicative. She is currently on an antidepressant, lithium (level 1.0) and risperidone 2mg nocte.

You are concerned that you may need to commence emergency ECT. Mary's husband and son however are strongly against ECT. They also report that Mary would be against ECT also and are against her being administered it.

15.1 Outline the ethical issues at play here

Beneficence versus autonomy - clear risks of further deterioration death, with effective treatment versus family's account of patients wishes. Lack of Advanced Health Directive and EPOA - relying on patients wishes

Inherent paternalistic ability of psychiatrists to determine whether one has capacity, and override autonomy with the oversight of MHA legislation

Non-maleficence - inherent risks of ECT - (anaesthesia, cognitive issues) that must be balanced against benefits

Duty of care – balance the risks and benefits of ECT – e.g. cognitive impairment vs. life-saving treatment etc

Justice – “busy metro hospital” – ECT provides a rapid clinical response and this may allow for further distribution of healthcare resources (for example will accelerate bed flow)

15.2 Explain to your registrar the evidence for ECT in this patient.

As per ECT guidelines:

- Meets criteria for ECT due to high risk of suicide given the family history and current mental state
- Has not responded to pharmacological therapies and unable to engage in psychological therapy as she is uncommunicative
- Not eating and drinking – therefore at risk of physical deterioration, even death, if there is no rapid response
- At risk of developing catatonia which also has a separate risk of death (i.e. malignant catatonia)

Requires emergency ECT and not routine ECT due to the rapid decline in physical health and the associated high risk of death with her current presentation – it will be a life-saving intervention

Indications for ECT include the treatment of the following:

- Depressive disorders including major depression and major depression with psychotic features, major depression with melancholic features and major depression with peripartum onset.
- Other psychiatric disorders such as bipolar disorder (manic, mixed and depressed phases), acute and chronic treatment-resistant schizophrenia, schizoaffective disorder, catatonia, acute psychosis, puerperal psychosis and neuroleptic malignant syndrome.

ECT should be considered for depressed patients who have not responded to adequate trials of medication and psychotherapy. ECT should also be considered when rapid clinical improvement is required. It is potentially a firstline treatment in severe depression when there is inadequate oral intake, a high suicide risk or high levels of patient distress, and also for patients with psychotic depression, acute psychosis, catatonia, delirious mania or previous positive response to ECT (Malhi et al., 2015). ECT is also used as a prophylactic treatment in major depressive disorder, bipolar disorder and schizophrenia through maintenance ECT.

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Mary continues to deteriorate and you decide to proceed with your plan to give ECT.

15.3 Outline your approach to giving Mary ECT

- Assess if valid consent is possible (unlikely due to current mental state). This includes serial observation and documentation of capacity as part of decision making process
- Family meeting to explain the decision to proceed with ECT. Ideally involve family with decision-making but explain duty of care under the MHA. Printed information for family and psychoeducation on expected side effects and treatment course. Consider follow up family meeting - to ensure adequate opportunity to address concerns and facilitate discussion re: treatment
- Second opinion by consultant psychiatrist required in most jurisdictions to determine if appropriate use and any alternatives to be considered
- Application to relevant MHA for emergency ECT, with consideration of additional local hospital requirement.
- Lithium: increases the risk of post-ECT delirium and should consider lowering the dose or WH pre-ECT
- (Not relevant to this question): Benzos and anticonvulsants should be reduced or withheld as these increase the seizure threshold
- No absolute contraindications to ECT but medical comorbidities should be considered and addressed appropriately - Medical work up and anaesthetic review - Medical history, medications, previous anaesthetic reactions/complications, bloods, ecg, hand dominance, vital signs to ensure safety of the procedure and no contraindications
- Consider type, pulsewidth, electrode placement in order to optimise treatment specific to Mary and her medical history/cognitive status - likely RUL with titration initially to minimise side effects
- Continue to monitor Mary's mental state and any adverse effects of ECT (e.g routine recall, attention and orientation). Monitor physical observations and ensure that they return to pre-ECT baseline.

MEQ 16 (RANZCP Aug 2015)

A 35yo man; James has been referred by his GP for assessment of Adult ADHD. James has been having difficulty holding down his jobs. He has a 10-year-old brother Clyde who has ADHD and is receiving treatment for this.

Q16.1 Outline how you would go about making a diagnosis of Adult ADHD.

Adult ADHD presents differently to child ADHD - inattention and organisational problems but less hyperactivity and impulsivity. Often comorbid with range of other disorders/features

Clarifying symptoms - longitudinally over several appointments with use of collateral

- Symptoms of inattention eg. not completing tasks, problems with work, distractibility
- Symptoms of impulsivity/hyperactivity eg. anger outbursts
- Rule out co-morbid psychiatric symptoms: high rate of co-morbidity depressive/anxiety/mania/hypomania symptoms, personality disorder, SUD
- Sleep dysfunction - possible cause of attention/ concentration issues

Functional history - determine impact of untreated illness, consistency with reported symptoms

- Work: employment history - description of difficulties re: holding down jobs, symptoms impact on functioning
- Relationship: current relationships and supports, difficulties as a result of symptoms
- Family: level of support
- Forensic issues - potential for diversion of stimulants

Targeted developmental and educational history - assessing for ADHD symptoms to meet diagnostic criteria or other psychiatric diagnoses better explaining presentation

- Developmental milestones
- Education history: specifically academic performance, any feedback from teachers regarding inattention/hyperactive symptoms/access to report cards
- Behavioural / conduct issues - age of onset, impact on family, forensic issues
- Attachment, communication and coping style
- Parenting style and coping strategies, substance use issues etc
- Trauma screen
- Personality assessment

History pertinent to presentation that might complicate treatment planning or be a contraindication to stimulant prescription

- Medical history: iron deficiency leading to poor concentration, anaemia
- Psychiatric history
 - History of past psychiatric diagnoses and treatments
- Substance Hx: Current and past substance use, gambling issues
- Current medications
- Family history
 - Clarify any family history of developmental disorders and further information about brother's diagnosis

Examinations to exclude comorbid illness and correlate with history taking

- Assess current mental state for signs of inattention or hyperactivity
- Neurological exam
- IV bruising from illicit use - potential for misuse
- Physical exam: BP, HR, weight
- ASRS Screening Tool
- ACDS diagnostic tool, Conners Adult structures I/V

Collateral to confirm aspects of presentation and specifically to clarify under reporting or substance use issues. Also to make diagnosis need historical records of report cards. Need to obtain collateral from at least one or more sources

- Report cards
- GP
- Parents
- Employer

James has had difficulties in the social and occupational aspects of his life. He had been using his brother's methylphenidate with good effect. He has lost jobs in the past due to drug use. He describe some minor driving accidents. He gives history of onset of symptoms in early primary school.

Q16.2 What are the main issues and how will you address them?

Diagnostic issues

- Diagnostic uncertainty requiring clarification and longitudinal assessment: ADHD vs substance use disorder vs other psychiatric differentials

Risk issues

- Risk misuse - polypharmacy of diversion - regular review and staged supply
- Risk of misadventure - caution against polypharmacy, side effects, risk of psychosis, overdose
- Driving risk - use of long acting and consider professional assessment

Management issues

- Assessment and treatment of substance use disorder. Use of random UDS
- Consideration for long acting agent or non -stimulant (reduced abuse potential)
- Staged supply and monitoring of dispensation, treatment contract ie no new scripts
- Assessment treatment response - collateral from family and employer

Ethical issues / Medicolegal

- Using brother's methylphenidate - legal issue of using someone else's prescription. Diverting medication away from brother - brother may be undertreated
- Professional reporting requirements (PBS, pharmacy). Consider contacting GP of patient's brother
- Contact DDU or relevant authority

You have been asked to give a talk to a group of GP's regarding Adult ADHD.

Q16.3 Outline what you would discuss. (In the exam they could also ask you the approach)/Discuss the key issues you would consider in your talk.

Can use CANMEDS

Preparation - to maximise effectiveness and appropriateness of talk,

- What staff / disciplines at each practice
- What is their knowledge level and personal experience
- Goals of teaching session -
- Modality of presentation - case based, didactic approach, slide presentation
- Principles of adult learning - involvement in discussing case studies

Psychoeducation about illness in order to raise index of clinical suspicion and minimise both over and under treatment

- Epidemiology and statistics

- Specifics on how adult ADHD differs from childhood ADHD: more prominent inattention and distractibility, can manifest in work and relationship problems
- Importance of clear history of persistence from childhood
- High psychiatric comorbidity especially substance use

Diagnosis - emphasising the need for triangulation and collateral needed for diagnosis

- Diagnostic criteria, retrospective history required, collateral from school/family
- Rating scales and education on implementation
- Often strong family history
- Exclude comorbid diagnoses

Management - discuss the options medication, psychological and lifestyle approaches

- General principles of ADHD medications: stimulants vs non stimulants, mechanism, indications, risks and monitoring, side effects, relative efficacy
- Prescriber rules and regulations - schedule 8
- Role of psychological therapy and practical strategies eg. diaries, reminders
- Psychoeducation (about the disorder, impacts and how to function optimally while having ADHD, Sleep management, anger, organisational skills, etc), behavioural (ADHD coaches, anger management etc) and or occupational interventions, social (social skills training etc) and psychotherapies (CBT, supportive counselling, relaxation therapy, family therapy etc); liaison with other health workers

Referral considerations - how the GP's can seek additional support for managing their patients and seek specialist opinion

- Psychiatry involvement
- Shared care process
- Psychology input
- Pharmacy

MEQ 17 (RANZCP Aug 2015)

Stan is a 20-year-old indigenous male. He has been admitted to the mental health unit after a violent assault on his father. His parents report that he has not been himself over the last 6 months and has been seen talking to himself on a number of occasions. He is also reported to smoke cannabis on a regular basis.

17.1 List the short and long-term risk factors you would assess for in this case that would increase the risk of violence?

Short term

1. Command hallucinations, misidentification, persecutory beliefs
2. Access to weapons
3. Poor sleep, level of agitation
4. Current Intoxication or withdrawal
5. Presence of delirium
6. Expressed homicidal ideation
7. Suitability of treatment setting (HDU vs open ward)
8. Under medicated / inappropriate treatment
9. Lack of security presence/assistance
10. Level of cultural appropriate care and support

Long term

1. History of aggression (static history)
2. Ongoing substance use
3. Insight into mental illness
4. Treatment compliance / level of engagement

5. Level of mental health and community supports
6. Personality factors
7. Cognitive impairment

Following your assessment of Stan an indigenous male, you diagnose him with schizophrenia. His medical investigations are normal. You find out that he has been admitted previously to another mental health unit where he was diagnosed with schizophrenia.

He was started on Risperidone 6 mg, which he stopped 9 months ago. He continues to complain of ongoing auditory hallucinations but refuses to take any medication.

17.2 Outline your approach to this situation.

1. Approach considerations to maximise safety and accuracy of diagnostic process including cultural considerations
 - a. Indigenous mental health worker present
 - b. Respectful approach towards Stan
 - c. Consider whether family would be useful or distressing
 - d. ? case manager present(or security if indicated)
2. Risk assessment - need to determine current risks to patient and others
 - a. static and dynamic factors,
 - b. Current level of agitation, delusions, command hallucinations
 - c. Environmental factors including setting, staff experience
3. Collateral - to improve understanding of relevant past history, family issues, and potential risk issues
4. Obtain information from the previous admissions, speak to past treating psychiatrist, review letters, Previous risk issues
5. Past treatments, doses, compliance and side effects
- 6.
7. Recent functioning, family relationships
8. Assess reasons for refusing medications in order to manage these concerns and prevent future relapse with non-compliance
 - a. Allow Stan to openly express himself
 - b. Ask about cultural reasons that may be impacting him (related to indigenous beliefs)
 - c. Assess for past side effects - EPSEs, sedation, sexual dysfunction, weight gain
9. Psychoeducation to assist in identifying triggers and warning signs of relapse and enhance efficacy in seeking treatment when unwell
 - a. Discussion on contributing and perpetuating factors
 - b. Chronicity of condition, trajectory of illness
 - c. Risk-benefit discussion about treatment vs non treatment
 - d. Education to family including - relapse prevention pathway
10. Capacity assessment to determine ability to provide informed consent or whether mental health act is required to minimise imminent risks
 - a. Evaluate Stan's understanding of his mental illness and whether he understands the risks of not taking medications
 - b. Assess for current risks (especially command hallucinations and thoughts to harm others as he has already assaulted his father).
11. Plan management seeking to maximise engagement and compliance, reduce risk of relapse
 - a. MHA consideration
 - b. Risk mitigation strategies - leave, level of observation, etc
 - c. Consider depot medication if compliance issues , consider webster packing if oral medication used

- d. Ongoing cultural liaison
- e. Referral to local drug and alcohol support

After a 4-week admission Stan is better. The family is concerned about his being discharged in the community as it is an indigenous community and his parents are worried about the impact of this on Stan's mental health. They do not want any community follow up from the CMHT, but want to go to the traditional healer.

17.2 How would you manage this situation?

- A. Arrange family meeting, Involving indigenous liaison / IMHW to facilitate cultural understanding and communication of clinical issues
- B. Explore explanatory model of illness, views on current diagnosis
- C. Exploration of historical factors in local community, and how these impacts on authoritarian treatment style
- D. Consider ways to minimize stigma through psychoeducation or confidentiality in treatment
- E. Clarify their specific concerns about Stan attending the community - is this stigma from community, access to substance use, other?
- F. Consider cultural background and gender of key stakeholders in community and how these people could contribute to recovery or impair engagement
- G. Provide psychoeducation and attempt to negotiate parallel treatment approaches, with cultural treatment strategies as adjunct to medication and community MH engagement
- H. Psychoeducation on the risks associated with relapse, both in terms of acute risks of aggression, rate of relapse, impact of cannabis on illness
 - I. Safety planning and relapse prevention pathway for family to empower them to seek further support if at risk
- J. Explore alternative models of care including GP/shared care with CMHT, telehealth reviews - this may better assist engagement and decrease the risk of relapse
- K. Capacity issues - review need for involuntary treatment. Cultural considerations and impact on therapeutic alliance
 - L. Determine need for depot antipsychotic medication in assisting with non-compliance and relapse

College feedback: MEQ 3 concerned an indigenous adult presenting with chronic paranoid schizophrenia. Parts of this MEQ were generally answered well, e.g. including the whole family. However, the sub-question on responding to the cultural worker's request was generally poorly done, with candidates focusing too narrowly on one aspect, e.g. the specific content of the request. A broader context for consideration of answers was necessary to score well.

MEQ 18 (RANZCP Aug 2015 and Feb 2018)

You are a rural psychiatrist. Your patient is a 52 yr old Indigenous man with chronic schizophrenia managed with flupenthixol depot 20mg imi 2 weekly. The man lives with his son, daughter in law and 2 grandkids (3 and 5 yr old). In the past when unwell, he has responded to an increased dose in his medications. He has a past history of verbal aggression and paranoia towards the neighbors. It seems he is deteriorating in his mental state at present. His son thinks this is because his uncle died recently.

18.1 Outline the risks inherent in this scenario.

- Risks to children present in house. Exposure to violence, direct risk of harm, neglect, incorporation into delusions
- Increased aggression risk given past history and current deterioration in mental state indicating relapse
- Further deterioration and disengagement - exacerbation of psychosis, increasing treatment resistance
- Vulnerability of being harmed if voicing psychotic beliefs
- Damage to reputation and family standing in community if acting on psychosis
- Risk of misadventure, criminal charges, detainment by police, substance use issues
- Risk of suicide - recent bereavement, possible suicide of brother
- Self neglect - reduced self cares, food intake, physical health issues

The son really wants his father to be managed at home. He believes he can convince his father to take some oral medications (Olanzapine). You agree to the plan.

The cultural liaison worker informs you that he believes this is a culturally normal grief reaction, rather than a relapse of Schizophrenia. He feels that the patient should not be managed with medication, that he should be managed with traditional healing instead.

18.2 Outline your management of this situation.

Risk - Need to generate a risk formulation and share this with relevant stakeholder. Communicate concerns in regards to risk of self, others and particularly children. Concern for carer burnout for son

Arrange a meeting with cultural liaison, relevant family and community persons (eg. elders) as relevant stakeholders needed to support treatment planning and action management plan

Explore cultural factors, model of illness and rationale for liaison workers concerns as this is important in diagnostic formulation and also in addressing family concerns and providing justification for the use of medications

Seek to develop a shared formulation - including biopsychosocial/cultural factors. Stress associated with bereavement likely has contributed to relapsing illness, however may not alone fully account for presentation

Specifically address IMHW opinion - emphasise past response to medication and risks associated with further deterioration, particularly emphasis on presence of children and impact illness has on them

Negotiate a treatment and follow up plan, ideally with concurrent use of medical and traditional healing methods - a shared management plan will be most likely to be complied with minimising risk of relapse

Clear pathway for escalation and more assertive community treatment, safety planning, crisis contacts in situation of deterioration

Consider additional supports such as social work, extended family support, NGO to support recovery activities with goal of social integration and improved community functioning

The patient appears to have become paranoid about the neighbours again. He has also been looking suspiciously at his grandchildren. His daughter in law no longer feels safe with him in the house and wants him out. The son Robert tells you about this and states he feels torn between supporting his father and wife.

18.3 Describe your advice.

- Validate difficult situation given dual role in supporting father and this own children
- Outline legitimate risks issues to children given inclusion in psychotic symptoms. The need to act on these imminent concerns whilst still supporting his father
- Discuss pathways to contain risk with clear priority for child welfare
 - Admission to inpatient unit vs increased community supervision, however the latter requires children be removed
 - Removal of kids from the house vs alternative accommodation for patient
 - Consider recommencement of depot medication given current non-compliance with orals
 - Use of MHA (if not already in place)
- Further family meeting outlining risk concerns to involved stakeholders. How can family/community support be mobilised to support Robert and his family?
- Suggest son maintains advocacy role - treatment team to take responsibility and blame for decisions made including restrictive practices
- Consider the need for support for the grandchildren given exposure to stressful environment, and likely need for relocation
- Social work review given potential accommodation issues and potential to refer family to additional NGO or community supports

MEQ 19 (Feb 2016 RANZCP)

You are the psychiatrist at a community mental health clinic. Jean is a 45 year old woman of indigenous background. She has a history of sexual abuse and now suffers from Post Traumatic Stress Disorder characterised by nightmares, hypervigilance and agitation. She has recently seen her assailant, who is actually an older relative at a family function. This has triggered her symptoms and caused her immense distress. You are contacted about this by Jean's case manager who is concerned about Jean's risk of self-harm.

Q19.1 Outline the information you would like to seek from the case manager prior to reviewing Jean. (8 marks)

1. Immediate information - to inform acuity and imminence of risks
 - a. Current location of pt and current risks of further family violence
 - b. Current mental state (distress, impulsivity, dissociative sx, psychotic sx)
 - c. Willingness for review
2. Risk -
 - a. Vulnerability risks - to further harm/victimisation
 - b. To self - presence of suicidal ideation and intent, past attempts/DSH
 - c. To others - homicidal ideation, threat of retaliation to perpetrator
 - d. Risk to reputation - standing in family/community, issues of shame or victim blaming, gender factors
 - e. Substance use (increases all risks)
 - f. Neglect of dependents, child exposure to mental illness and violence
3. Cultural factors that might influence assessment and management
 - a. Involved Indigenous Liaison worker

- b. Stigma and attitudes to health service, family attitudes
 - c. Belief in traditional healing, explanatory model of illness
 - d. Previous community response to abuse, QPS involvement etc
- 4. Supports - able to be mobilised as part of treatment plan with goal of mitigating risk and enhancing recovery
 - a. Level of family support - protective response
 - b. Living situation - alone or with others, with dependents (considering child risk)
 - c. DV services or other NGO
- 5. Clinical factors
 - a. Previous stability of mental state and functioning
 - b. Diagnoses, PTSD symptomatology, nature of trauma so can minimise chance of retraumatization
 - c. Previous treatments and responses, past hospitalisations - indication, duration, effectiveness
 - d. Usual method of coping distress (maladaptive or self-endangering behaviours)
 - e. Current medications (compliance, antidepressants, antipsychotics, sedatives)
 - f. Previous psychological interventions - engagement and response

Jean states she feels better now, she denies intent to harm herself and tells you "I just want to go home so I can get some sleep".

Q19.2 Outline your advice to the case manager on how to manage Jean. (8 marks)

1. Recognised psychiatric symptoms of mental illness may not be expressed readily and high index of suspicion is required
2. Clarify the complaint, symptoms of depression, PTSD, insomnia and substances
3. Psychoeducation on sleep disturbance, antidepressants and PTSD, risk of dependence of benzodiazepines, metabolic effects of quetiapine
4. Risk and ethics
 - a. Capacity assessment - willingness to engage
 - b. Document risk assessment
5. Psychoeducation around early warning signs and response plan including risk modification strategies
 - a. Limit access to means (remove extra meds from home)
 - b. Mobilise family supports (can family or friend stay with her o/night)
 - c. Psychoeducation around minimising substance use or alcohol as increases risk
 - d. Safety plan - what to do if SI returns, discuss after hours support (crisis line, ED presentation after hours)
6. Arrange f/u appt tomorrow with IMHW with risk assessment given unresolved stressors
7. Offer family liaison/meeting if patient amenable
8. Exploration of need for practical support, or additional service input
9. Consider arranging staged supply if overdose potential is deemed high

(Specific treatment for sleep - not advice to case manager)

10. Symptom control for sleep
 - a. Characterise sleep pattern and difficulties
 - b. Psychoeducation that long-term mgmt of sleep should focus on behavioural interventions but acknowledge use of short term pharmacological strategies in crisis
 - c. Consider appropriate class and duration. Initially frequent review with goal of short term use only
 - i. Benzo's long acting (diazepam) - caution in substance use
 - ii. Low dose quetiapine (off label)
 - iii. Sedating antidepressants (mirtazapine)

- iv. Consider prazosin or clonidine - slow titration with blood pressure monitoring
- v. Zopiclone or zolpidem - not on PBS
- vi. Melatonin

Jean has told her case manager that she has had great relief just from talking. She would like to inform her daughter of her history of abuse, however she is fearful that her son or others in the community may find out.

Q19.3 Outline your advice to the case manager on how to manage Jean. (6 marks)

1. Ethical and Legal issues
 - a. Autonomy - right to make own decisions , confidentiality and privacy in close knit community with risk of stigma
 - b. Risk of harm of disclosure - community backlash
 - c. Mandatory reporting -consider the need if child risk issues are present,
 - d. Discussion with clinical director or peer supervision for advice if unclear
2. Obtain further information - involve IMHW who can facilitate understanding of cultural nuances
 - Reason wants daughter to know but not her son
 - Clarify age of daughter, level of maturity, coping style - capacity to manage such disclosure
 - Daughter's own mental health- potential for exacerbation
 - Decisional balance based on above - acknowledging autonomy vs potential harms to child
3. Planning for disclosure
 - Right place and time - consider the need for additional support (consider family, case manager or community assistance)
 - Follow up for child as needed
4. Assist in arranging further psychological support for mother
 - Community elders and healers
 - With case manager if clinical experience/expertise is appropriate (at timing permits)
 - Referral to trauma focussed CBT
 - Peer groups / survivor groups of domestic violence

MEQ 20 (Feb 2016 RANZCP)

You are a CL psychiatrist. You are referred Bill by the endocrinology team. Bill is a 45 year old married electrician who has a history of pituitary carcinoma. His tumour was successfully removed by the endocrinology team. However since that time, Bill has developed mood lability, social withdrawal and low self esteem. He initially returned to work as an electrician, however, left his work after six months as he had lost his confidence. He also suffers from chronic headaches and insomnia.

Bill has sleep apnoea and sleeps with a CPAP machine. He is managed with thyroxine and corticosteroids.

Q20.1 Outline the possible reasons for Bill's mood lability (6 marks)

1. Organic/medical
 - a. Frontal lobe pathology - secondary to trauma, scarring, ischemia,
 - b. Steroids - capable of adversely affecting mood
 - c. HPA axis - pituitary removal - altered thyroid and cortisol levels impacting mood and affect
 - d. Delirium - hypoactive delirium with depressive features
 - e. Chronic pain syndrome - leading to demoralisation, low mood and irritability
 - f. Circadian rhythm dysfunction impacting mood- CPAP or hypoxia/non-compliance with CPAP
2. Psychiatric (new or exacerbation of pre-existing)
 - a. Adjustment disorder - adjusting to diagnosis, loss of functioning
 - b. Depression
 - c. Anxiety - health anxiety, mortality salience, somatisation
 - d. Substance use disorder and withdrawal (e.g. EtOH)
3. Normal grief reaction to loss of role and function as a result of illness
4. Contribution of social factors
 - a. Financial stress
 - b. Relationship stress with wife

You meet with Bill and his wife. Bill's wife has taken over cleaning, shopping and management of the bills since Bill's surgery. Bill feels angry and frustrated about this. According to the Occupational Therapist, Bill is capable of doing these things himself.

Q20.2 Outline your management of this situation. (8 marks)

Assessment of medical contributions - organic causes

- Review recent investigations - thyroid levels, cortisol levels, pain management and possible side effects of medication side effects

Assess for psychological comorbidity that may be treatable and contributing to presentation

- Mood disorder
- Anxiety disorder
- Cognitive examination (completed by OT)
- Personality style

Collateral - wife, medical teams, G.P employer to ensure consistency of reports and accurate diagnostic picture

- Premorbid personality and coping
- Level of engagement with recovery
- Trajectory of decline, precipitants
- Compliance with CPAP
- substance use issues

Exploration and management of dynamic considerations to manage Bill's distress and support wife in caring for Bill

- Understand previous marriage dynamic and roles, and disruption of this by surgery
- Change of role with progressive adoption of sick status
- Facilitation of this by well meaning wife compensating for husbands functional decline
- Communication dysfunction

- Assess for carer burn out

Management aimed at maximising physical and psychological recovery

- Psychoeducation on the psychological factors propagating further dysfunctions
- Treat mood and anxiety
- SSRI - some evidence for citalopram in frontal lobe pathology
 - Psychological therapies such as CBT, however acknowledge difficulties in frontal lobe dysfunction
- Gradual reintroduction of structure and routine responsibilities with ongoing review and OT involvement
- Support for wife - family supports, counselling, psychology or carer groups, respite

The endocrinology team tell you that Bill's tumour has returned, although it is treatable. Bill is becoming increasingly irritable and labile in his mood.

Q20.3 Outline the possible causes for Bill's mental state deterioration. (4 marks)

1. Neuropsychiatric manifestations:
 - a. Chemotherapy side effects - neurotoxicity
 - b. Space-occupying lesions - direct brain involvement
 - c. Re-commenced steroid medication - linked with mood lability and psychosis
2. Psychological defences/reactions:
 - a. Demoralisation
 - b. Anger directed at treating team for perceived treatment failure
 - c. Denial of illness
 - d. Fear of death
3. Secondary psychiatric manifestations:
 - a. Adjustment disorder relating to diagnosis
 - b. Major depression
 - c. Self-regulating mood with substances
4. Relationship conflict
 - a. Arguments around treatment or financing expensive treatment
 - b. Fear of loss of spouse

MEQ 21 (Feb 2017 RANZCP)

John 12 yo ADHD & now Washing hands with increased frequency and arguing with family by also pressuring family to wash hands also (developing OCD)

Q21.1.What information do you need to complete the assessment (12)

1. Risk Assessment
 - a. Risk to self - levels of distress, risk of inadvertent self harm from handwashing
 - b. Risk to others - aggressive behaviour to family?
 - c. Risk of educational interruption
2. History of ADHD & OCD in order to establish duration of illness, past treatments
 - a. Clarify their temporality (which occurred first)
 - b. Clarify how the diagnosis was made - ensure correct diagnosis
 - c. History of treatment and response (eg drug, dose, compliance, adverse effects)
 - d. Primary disorder vs secondary OCD arising out of stimulant use
3. Focused hypothesis based interview & MSE exploring differentials:
 - a. ADHD - hyperactivity/fidgeting, poor attention, unable to play quietly, excessive talking/blurting out answers

- b. Anxiety - separation anxiety (maintaining proximity to family), behaviour around family members, sleep problems, excessive reassurance, poor school performance, somatic symptoms (stomach ache)
 - c. Tourette's - commonly comorbid with ADHD, repetitive motor tics that could mimic compulsive behaviours (eye blinking, facial grimacing, body gyrations, obscene gestures, obscene words, palilalia)
 - d. Pervasive developmental disorder/ASD (ADHD is a developmental disorder) - poor social communication, poor emotional reciprocity, stereotyped behaviours, restricted interests, insistence on sameness
 - e. Recent Infections - PANDAS
 - f. Medication induced post stimulants for ADHD
- 4. Other contributions that might be included in formulation in order to guide management
 - a. Exploration of broader social context and stressors (parent relationship/school/ recent illness in family)
 - b. Family response to developing situation
 - c. Effectiveness and consistency of behavioural interventions
 - d. FHx of mental illness - anxiety, tourettes, OCD
 - e. Family attitudes to handwashing - e.g. role modelling from parents

He is diagnosed with OCD.

Q22.2 What would be your management?

1. Explore parents views on illness, management preferences, attitudes to medication
2. Communication
 - a. Start with patient - engage in empathetic manner to build rapport and alliance
 - b. Psychoeducation - in age appropriate manner, minimize blame (not their fault)
3. If stimulant related - consider dose reduction or change to atomoxetine
4. Med expert/OCD treatment:
 - a. Consider setting - frequent outpatient reviews, home visits or planned admission to inpatient unit
 - i. CBT - exposure and response prevention
 - ii. Family psychoeducation - and age appropriate resources for pt and siblings
 - iii. Clear advice to avoid collusion as this acts as a perpetuating factor
 - iv. Give permission to the family, with pt present, to not engage in this behaviour (get everyone onside not to collude)
 - v. Consider high dose SSRI if poor response to above by need to monitor for self-harm and SI. Consider clomipramine - however more prominent side effect profile

Q23.3 How can you support the family?

1. Social work review to consider external sources of support - carer or parental support groups, Centrelink/financial issues
2. Family therapy - do siblings require input or specialised support
3. Involve the school with family support - less school issues and therefore less associated stress for family. Share behavioural intervention plan to manage compulsions
4. Consider psychological support for other family members
5. Ongoing Psychoeducation, printed information, websites

MEQ 24 (RANZCP Feb 2017 and August 2018)

4yo Kelly had surgery to remove intrathoracic mass. She has just returned from surgery & the pathology advised the mass is now found to be malignant. Father behaving verbally aggressive to staff, hallucinating, acting oddly, shouting and refusing to allow the child's bloods to be taken. He's declined further blood tests for Kelly. You are CL psychiatrist and get a call to come.

Q24.1 Outline the information you need from the treating team (10 marks)

1. Child's safety currently in order to determine acute management such as need for security and medium term what interventions are required
 - a. How urgent is blood-test for child
 - b. Is the child medically stable? What's her diagnosis and prognosis?
 - c. Is she distressed or in danger?
 - d. What treatments are available to her?
2. Acute risk assessment in order to better manage father's behaviour with external means such as security or facilitate deescalation with family
 - a. Threatening or abusive - level of risk to staff or child
 - b. Summary of behaviours exhibited
 - c. Evidence of intoxication from substances
 - d. Do they need security to ensure staff safety
 - e. Environmental considerations - visibility of patient, low stimulus, proximity to other vulnerable patients
3. Outline of events in terms of daughter's diagnosis - this will assist in de-escalation of father and give some ability to empathise and build rapport
 - a. Timing and manner in which he was informed of dx
 - b. Father's level of understanding of situation
 - c. Previous behaviour/past interactions with staff
 - d. Other family members involvement /ability to support father
4. Communication/cultural considerations of father that might have precipitated outburst or made de-escalation more challenging
5. Guardian/substitute decision maker- required to consider the legal and ethical aspects of the case and how management will proceed
 - a. Is he the legal guardian or is there another legal guardian present (may help with decision making but also de-escalation)
6. Father's Hx & current intervention - this would inform formulation of his presentation and what risks need to be addressed for father and child
 - a. Known past MH Hx?
 - b. Any indications of substance misuse?
 - c. Has verbal de-escalation been attempted?
 - d. S/W involvement - past child protection issues

Q24.2 Differentials for father (8 marks)

1. Acute stress reaction / brief reactive psychosis /dissociation
2. Substance intoxication/withdrawal
3. Relapse of major mental illness - schizophrenia/BPAD
4. MDD with psychotic features
5. Adjustment disorder

6. Anger secondary to treating team factors (poor provision of information, insensitive delivery of bad news)
7. Poor response to stressors due to personality factors / disorder
8. Cultural based reaction to grief

Q24.3 What are the ethical considerations around management (8 marks)

1. Competing duties of care - to medically unwell child vs mentally unwell father
 - a. Duty to child usually comes first - may need to remove the father if he is considered as putting daughter at risk
 - b. Beneficence versus autonomy of father
 - c. Provision of a safe environment for staff
2. Child protection obligations to the child
 - a. Mandatory reporting to child safety as there is a risk of harm and mother cannot protect child
3. Mental health act considerations - does the father need to be detained due to mental illness
4. Legal considerations - do the police need to be called and if so what is the implication for father
5. Consent & guardianship issues
 - a. If father is legal guardian and deemed not to be acting in pt's best interests, do emergency guardianship procedures need to be initiated for short term decision making?
 - b. Is there another substitute decision maker that can consent to patient's care?
6. Privacy and confidentiality - more likely to be related to father's condition
7. Justice - father deserves mental health treatment and intervention

MEQ 25 (RANZCP Feb 2017)

35 year-old medical officer in ED referred by GP for admission with a 5 week history of low mood, sleep problems, issues with concentration. GP states two past episodes of depression which were treated with antidepressants (8-9 months each time). He also noted that recently she had a complaint from a patient in ED and had to take a week off.

Q25.1 List and explain the information you need for a management plan.

1. Information gathering to understand the context of presentation and steps taken in resolving the complaint
 - a. Timing of recent stressor in relation to deterioration ? temporal relationship
 - b. Nature of complaint - involvement of APHRA, consequence for employment
 - c. Concern for impairment / performance issues in order to consider need for reporting or supervision
 - d. Involvement of medico-legal support for patient to facilitate recovery
2. Current Ax - assessing potential perpetuating factors, factors that might impede adherence to treatment/recovery
 - a. Full psychiatric history / treatment history
 - b. Medications hx- past antidepressants – dosage, class of drug
 - c. Current substance use
 - d. Psychosocial stressors (e.g. relationships, finances, work pressures)
 - e. Level of insight and judgement
 - f. Personality and coping style
3. Recurrent Depression Hx - informs prognosis and management of the illness and some indication of risks to self and of relapse
 - a. Compliance with antidepressants - explains poor response?
 - b. Type and dose of antidepressants - different classes trialled, at adequate dose?
 - c. Comorbid medical disorders (Coronary artery disease, thyroid issues) - unlikely but increase chance of tx resistance
 - d. Augmentation strategies if tried
 - e. Other associated factors that may be contributing to depression (e.g. chronic pain)
 - f. Comorbid psychiatric illnesses (e.g. substance use hx, bipolar illness?)
4. Risk/Manager - primary goal is to ensure safety of patients and the community who may be influenced by their care. Goal is to attempt to mitigate dynamic risk factors as part of management plan
 - a. Suicidal ideation, past hx of suicide attempts
 - b. Risk of harm to patients due to impairment from illness
 - c. Risk to reputation and finances
 - d. Protective factors/additional supports - family, colleagues, doctor support organisations
5. Professionalism - legal or ethical duties required as part of management of a complex case
 - a. Need to self report if impairment
 - b. Mandatory reporting if significant risks
 - c. Speak with your medico-legal team /indemnity insurer
6. Confidentiality is essential due to potential impacts on patients career as well as a general principle of treatment however need to discuss in which circumstances this is breached
 - a. Make sure pt is aware of reporting requirements
 - b. Setting - may be a risk to confidentiality from medical records or co-workers in public system
 - c. Consider private admission or out of catchment admission

During her admission she tells you she has had 2 episodes of one week duration in the past where she did not sleep, felt lots of energy & her friends said she was not herself.

Q25.2 List and justify how your management would change given this new information?

1. Given new information consider revision of diagnosis to BPAD1/BPAD2 and review psychiatric history and more thorough risk assessment. This is required and a mood stabilizer may be needed and also has different implications for follow-up and recovery
2. Risk now include depressive and manic phase of illness and as such require further review
 - a. Obtain further information of past episodes in regards to risky behaviour - personal, financial, reputational as well as triggers for illness (poor sleep, stress, substances)
 - b. Would revise risk formulation to include mixed or elevated states which increase risk of suicide
 - c. Additional of vulnerability risks and reputation risks, especially given the stressful nature of the job and potential to impact career prospects
 - d. Increased propensity for impaired capacity in doctor role in manic episode due to being impulsive and perhaps grandiose
 - e. Revise safety planning to include family/spouse/friends in detecting early warning sign of mania
3. BPAD:
 - a. Psychoeducation on new diagnoses including information on triggers for relapse, the importance of managing biological and social rhythms can help to better manage treatment
 - b. Initiate mood stabiliser - with pt's consent after discussion of risks vs benefits
 - c. Consider reduction and cessation of antidepressant as may contribute to manic switch
4. Medication specific considerations:
 - a. Not to consider valproate in women of childbearing age - risk of fetal malformation
 - b. Limited evidence of lamotrigine for preventing manic episodes therefore may be ineffective
5. Address potential complications or adverse effects of giving treatment
 - a. ?PCOS
 - b. Thyroid
 - c. Kidney dysfunction
 - d. Steven Johnson
 - e. Weight gain
6. Notification/confidentiality concerns:
 - a. Consider if notification of impairment is necessary, if so discuss with pt and preferably make notification in front of them
 - b. Encourage pt to talk to medico-legal insurer to better be able to assist if complications arise
 - c. Encourage pt to notify AHPRA and boundary for doctor notification if evidence of significant risks to patients
 - d. Aim to maintain therapeutic alliance despite the above which may be challenging in many situations

The Director of ED call you to discuss her admission. She also tells you of a complaint made by a patient against the medical officer for acting inappropriately.

Q25.3 List and explain the ethical and work issues at play in this example

1. Dual agency dilemma - duty to patient however also duty to protect community
2. Confidentiality vs beneficence - a duty to protect privacy yet balance this against harms to others
3. Beneficence
 - a. Is it in the best interests of the patient to talk to the director whilst unwell - may jeopardize what is said and consequences
 - b. Risk of complaint prompting deterioration in mental state
 - c. Risk to patient's reputation
4. Confidentiality
 - a. Speak to patient before giving director any information
 - b. Advise pt of phone call with director
5. Professionalism/Therapeutic alliance

- a. Speak with medico-legal representative to ensure best possible resources/up to date if needed to go to court
 - b. Advise pt to consider self-report and assertive management of the situation
 - c. Inform of mandatory reporting obligation - due to concerns of harm to patients (offer to make the report with the patient) to better facilitate this
- 6. Clinical governance issues
 - a. Advocacy for supported return to work
 - b. Training - have support procedures been in place
 - c. Work stress - has there been adequate support and supervision
 - d. Patient interests (right to supported employment) vs capacity of ED service to manage risks and relapses

MEQ 26 (9.1 RANZCP)

You are a consultant of a community mental health clinic. Your patient is a 39 y.o woman with a 10 year history of chronic schizophrenia. Despite good adherence to medication, she continues to experience distressing auditory hallucinations. She is prescribed oral risperidone 6mg nocte. She complains of EPSE. She requests a change of medication.

(REFER TO RANZCP AUGUST 2018 Q1)

MEQ 27 (RANZCP Aug 2015)

You are working as the junior consultant in a large general hospital. Tony, aged 23 years, was involved in a motorcycle accident in which he sustained multiple internal injuries and an unstable neck fracture. Tony has surgery to stabilise his cervical spine and is currently immobilised in halo traction, has a chest drain, and cannot be moved. Two days after the surgery, the spinal injury team contacts you expressing concern at Tony's excessive demands for opiates. When you speak to Tony he tells you that he is 'in agony' and can't sleep because of the pain in his back. He looks anxious and low in mood, but denies anxiety or depressive symptoms.

Tony is demanding to smoke cigarettes and cannabis and says that he cannot cope with hospital without them. He has threatened to sign himself out of hospital. He is verbally aggressive towards staff especially when they try to discuss the hospital smoke-free policy with him. The staff would like your help in managing Tony.

Your registrar says the diagnosis is Antisocial Personality Disorder.

(REFER TO RANZCP AUGUST 2015 Q1)

MEQ 28

You have recently assessed Mr Newbery, aged 84 years, who lives in a residential care centre for people with dementia. Mr Newbery suffers from moderately severe Alzheimer's disease.

The staff are concerned that he is verbally aggressive towards staff and other residents, and occasionally lashes out and hits staff. Some of the untrained staff in the centre feel that Mr Newbery is deliberately "playing up" and are annoyed with him.

When you examine Mr Newbery he is calm but severely cognitively impaired. He has no apparent physical problems. Recent routine laboratory investigations including mid stream urine culture are unremarkable. Mr Newbery is on no medication.

(REFER TO RANZCP AUGUST 2017 Q2)

MEQ 29 (Feb 2016 + Aug 2017)

John is a 55 year old principal of a local school who is referred to you privately, by his general practitioner with a history of BPAD... he is currently managed with Sodium Valproate 1600mg and Olanzapine 20mg. He is a successful teacher and lives with his wife and three children. He has gained a lot of weight on his current regime (over 10kg) and is upset about this & would like to trial a different medication regimen

(REFER TO RANZCP AUGUST 2017 Q5)

MEQ 30

You are the Junior consultant psychiatrist of a community mental health clinic outreach to Wirdajuri. Wirdajuri has a population of 3,000 & you fly in once every 3 weeks for a clinic/3 days per month.

Donna is a 42/54-year-old Indigenous woman referred by visiting GP who has presented in extreme distress with anxiety, not sleeping and has developed fear of leaving the house in the context of her rapist who was convicted has been released from prison after 12 years & she is fearful of seeing him back in her community. She has contacted police about her concerns but has not found this helpful.

She was diagnosed with anxiety, depression & PTSD 11/2 years ago but was lost to follow up.

She is not suicidal, psychotic or depressed & is otherwise in good physical health and not on any medications.

(REFER TO RANZCP AUGUST 2017 Q1)

MEQ 31

You are the on-call consultant. Your registrar calls you about Mr Morris, 84yo man brought in post hanging attempt now medically cleared. Mr Morris indicates he thought the attempt was rationally based & is disappointed it did not work.

You are unable to access collateral from GP & family at this time.

Your registrar would like to admit Mr Morris under MHA.

(REFER TO RANZCP AUGUST 2017 Q4)

MEQ 32

32yo successful accountant, well respected at work, attends follow up soon to be made partner of the accounting firm. Married with 2 young children, wife cares for children at home. Recent overdose of paracetamol, full recovery. Helps/volunteers troubled teens in the community on his weekends.

Wife brings him in for follow up post overdose & states she is worried about increased use of adult pornography.

(REFER TO RANZCP AUGUST 2017 Q3)

MEQ 33 (Feb 2016 RANZCP)

You are the consultant psychiatrist treating Paul, aged 51 years. He was admitted ten days ago to the acute psychiatric unit in which you work with a diagnosis of Major Depressive Disorder with psychotic features. He was commenced on fluoxetine 20mg daily one week prior to admission and risperidone 3mg daily was added on admission. Paul's mental state had begun to improve and the level of nursing observation was reduced.

When you arrive at work early on Monday morning, you are told that Paul hanged himself in a ward bathroom during the weekend and that attempts to resuscitate him had been unsuccessful.

(REFER TO RANZCP Scoring Key MEQ 1)

MEQ 34 (RANZCP Feb 2016)

You are the consultant psychiatrist who is asked to see Adora, a 26-year-old Filipina woman who emigrated 5 years ago. She is married to Jeff and they have a 4-year-old son, Ben. Jeff works on an offshore oil rig and is away from home two weeks each month.

Adora has few friends and no family here for support. She often finds it difficult to manage Ben on her own. Ben has been having difficulties getting to sleep. There is conflict between Adora and Jeff over various things when he is home.

Adora was brought to hospital after she had been observed to be having a fit at her local supermarket at lunchtime.

The emergency department registrar believes Adora is experiencing 'pseudo seizures' and refers her to psychiatry for assessment and management.

You are the consultant who is called by your registrar to discuss the case after their assessment.

(REFER TO RANZCP Scoring Key MEQ 6)

MEQ35 (RANZCP Feb 2018)

You are a consultant working in the emergency department of an inner city public hospital. Your registrar calls to inform you that a 45 year old, single man has presented with suicidal ideation to the emergency department.

The man reports that he was recently charged by the police due to sexual assault of a co-worker whilst intoxicated.

The man is highly distressed.

(REFER TO RANZCP FEBRUARY 2018 Q4)

MEQ 36 (RANZCP Feb 2016)

You are the psychiatrist at a community mental health clinic. John is a 38 year old man who used to work as an electrician, but is now unable due to his OCD. He lives with his elderly parents. He has a long history of OCD characterised by obsessions around hygiene and checking rituals of taps and doorknobs. His parents do most things for him including shopping, cooking and cleaning. He has had a long history of erratic involvement with mental health services.

Q36..1 Outline the possible reasons for his poor engagement with mental health services. (6 marks)

1. Service factors
 - a. Mistrust of service
 - b. Unable to attend appt on time due to OCD
 - c. Transport difficulties/location
 - d. Previous use of coercive treatments - rupture alliance
 - e. Model of service, biased towards high risk illnesses
2. Clinician factors
 - a. Lack of therapeutic alliance or rapport with clinician
 - b. Lack of knowledge/experience with OCD
 - c. Therapeutic nihilism or countertransference
3. Individual factors
 - a. Felt stigma based on previous negative experience
 - b. Difficulty tolerating environment or transport to service
 - c. Lack of insight into level of dysfunction/impairment
 - d. Medication compliance - complex regimen, distressing SE's, poor response/efficacy, cost
 - e. Substance use
4. System/family factors
 - a. Poor parental insight - minimising problems
 - b. Enabling OCD behaviours
 - c. Negative views of mental health

You conduct a home visit and meet with John. He denies problematic substance use or symptoms of depression. He is agreeable to commence treatment as long as it doesn't involve admission.

Q36..2 Outline your psychological management of John. (8 marks)

1. Psychoeducation on OCD
 - a. Normalise intrusive thoughts - occur in everyone.
 - b. Thoughts are egodystonic and often experienced with negative consequences of inaction
 - c. Compulsions - maladaptive efforts to remove intrusions and prevent feared consequences
 - d. Explain negative reinforcement: compulsions temporarily reduce obsessional distress, perpetuating the behaviour
 - e. Compulsions/rituals then can trigger more unwanted intrusive thoughts
2. Assess for complicating factors that may affect engagement with psychological therapy:
 - a. Poor insight
 - b. Severe comorbid psychiatric illness (depression, anxiety, substance abuse, personality disorder)
 - c. Impaired cognition
3. Y-BOCS: measure of current symptom severity & response to treatment
4. CBT
 - a. Exposure and response/ritual prevention
 - i. Educate on strategies for distress tolerance and grounding (breathing/pmr etc)
 - ii. Early identification of thoughts and compulsions with guided tolerance
 - iii. Eliminate avoidance through repeated exposure to increasing gradient of distressing situations (from visualisation to real trigger)
 - b. Cognitive techniques:
 - i. Challenge the patient's appraisal of the obsessional thoughts meaning (socratic

questioning)

- ii. Promote alternative hypotheses to dysfunctional thought pattern
- iii. Continue to promote insight

Q36.3 Outline your pharmacological management of John. (4 marks)

1. High dose antidepressant - SSRI first line, or clomipramine
 - a. Trial for at least 6 weeks at therapeutic range before concluding drug is ineffective
 - b. SSRI therapeutic range can be double that of depressive dose - e.g. fluoxetine 40-80mg
 - c. Discuss risk of SE's (GIT esp., serotonin syndrome)
 - d. Check sodium in 2-4 weeks
2. Can augment with low dose antipsychotic if needed - e.g. olanzapine
3. Beware if pt is suicidal then clomipramine is contraindicated (due to toxicity in overdose)

The case manager informs you that John's father died.

Q36.4 Outline the possible consequences for John. (6 marks)

1. Psychosocial stressors:
 - a. Pressure on mother-alone to support him
 - b. Loss of income from financial support of parent
 - c. Possible loss of accommodation if mother cannot manage him
 - d. Transport difficulties - difficult to attend appointments for f/u
2. Exacerbation of OCD symptoms under stress
 - a. Intrusive thoughts? (feelings of responsibility for harm and mistakes)
 - b. Lack of closure of suppressed feelings of anger towards father
3. Grief, bereavement, depression due to loss of primary relative
4. Adjustment disorder with depression +/- comorbid anxiety
5. Improvement in OCD symptoms
 - a. Removal of enabling effect of father could result in increased 'exposure'-type therapy with associated remission

MEQ 37 (RANZCP Aug 2016)

Maria, aged 25 years, has a history of epilepsy since early childhood with multiple seizure types, a poor response to anticonvulsants, and longstanding cognitive difficulties (her full-scale IQ is 65). She lives with her parents in a small country town, and intermittently attends a supported work program packing boxes of fruit. She was admitted in status epilepticus to a tertiary intensive care unit and was ventilated for 3 days. Two days ago, she was transferred to the neurology ward. Since waking up she has been telling her parents that "everyone is spying on me with satellite cameras," and that the nursing staff are saying horrible things about her. She is worried that the food is poisoned. You are asked by the neurology team to see Maria.

Question 37.1 Discuss the information you would seek prior to seeing Maria. Please give reasons for your answers. (10 marks)

- Clarify referral - if concern is for behavioural management, diagnostic assessment or liaison
- Families level of involvement and concerns
- Review current file and discussion with medical/nursing staff
 - Recent investigations - vital signs, electrolytes, EEG, MRI, urine MCS
 - Current medications and those used for sedation/intubation - half lives
 - Current diagnosis/prognosis
 - Clarify problem behaviour/risks such as violence, non-compliance, absconding, verbal abuse
 - Review fluctuations in notes suggestive of triggers, delirium - any behaviour or sleep chart
- Current management
 - Use of pharmacotherapy
 - Behavioural interventions
 - Nursing special/restraint
 - Impact on staff - countertransference/burnout
- Past history - GP or family
 - Hx mental illness - any psychosis or mood disorder
 - Substance use history
 - Baseline functionally, premorbid personality and coping
 - FH mental illness
 - Seizure control - any post ictal phenomena
 - Exposure to trauma
- Social history/supports
 - Level of independence
 - Family dynamic/relationship issues
 - Access to NGO/NDIS/Respite
 - Family coping
 - Access to specialist services rurally – telehealth

Question 37.2 Give your differential diagnoses with a reason for each diagnosis. (6 marks)

- Postictal/Interictal psychosis
- Delirium
- Post ICU syndrome
- Ongoing temporal seizures
- Primary psychotic illness
- Acute stress reaction/Abnormal illness behaviour

The neurologist is happy for Maria to remain under her care as the risk of further seizures is quite high. However, the nursing ward staff have now become anxious about managing Maria on the neurology ward because they feel she is “crazy” and may be a danger to the other sick patients.

Question 37.3 Describe how you would address the nursing staff’s concerns. (7 marks)

- Validate nursing concerns for staff and other patients
- Arrange MDT meeting on the wards with involvement of medical and psychiatric staff, nursing and family/Maria where appropriate
- Identify specific concerns nursing staff have and what they see as an appropriate management response
- Outline ongoing CL Psychiatry review, contact details and ability escalate concerns (after hours registrar, indications for security). Daily psychiatry review
- Ensure confidence and knowledge of management plan and medications/behavioural responses, appropriate de-escalation strategies
- Psychoeducation that this is safest environment for Maria, the nature of the condition
- Offer inservice to assist in further development of skills

MEQ 38 (RANZCP Aug 2016)

36yo man developed schizophrenia at age 18yo. His case manager asks you to reassess him & do a report to support the extension of his 4th CTO. You are new to the team & this case manager has known him for only 2 months. He's only had one index admission for diagnosis, been on depot & orals for 4 years & now works at a radio station while living with mum & dad.

Q38.1 What questions do you ask the case manager before you see him?

- Assess how much involvement the new case manager has had in last two months and who the previous case manager was that may be able to provide more information
- Allow case manager to provide historical information
 - Social history - relationships with parents and more information about his job
 - Psychiatric information
 - Admission information - psychotic symptoms he experienced and what risks there were with respect to needing CTO
 - Engagement and treatment compliance when voluntary
 - Co-morbidities - any mood, anxiety, or substance use disorders
 - Medication history - which depots and orals currently and in-past, any side effects or concerns with medications. Compliance - current and historical
- Mental state examination information
 - Current psychotic symptoms and over the recent months
 - Has he been stable or fluctuating?
 - Any mood symptoms, decreased reactivity
 - Any suicidal thoughts
 - Negative symptoms - such as lacking self cares etc
 - Cognition / previous assessments
- Mental health act information
 - Capacity information related to insight and understanding of schizophrenia and views on the need for treatment
 - Risk information - any command hallucinations, thoughts to harm himself or others
 - Is there a less restrictive way to manage him (e.g. parents as guardians)

You decide to see the patient & organise the report.

Q38.2 What specific questions would you ask him in this interview?

- Review of symptoms
 - Review of psychotic symptoms - assess for hallucinations, delusions, thought content. High risk phenomena including commands and misidentification
 - Mood and anxiety symptoms
 - Substance use
 - Neurovegetative functioning
 - Cognitive assessment
- Review of social/occupational/relationship functioning/current stressors
- Capacity assessment
 - What is his understanding of his mental illness
 - Does he understand the treatments - benefits vs risks
 - Decisional balance
 - Is he able to make an informed decision about his illness
 - Available supports to assist capacity and decision
 - Stability of capacity

- View treatment and the use of CTO
- Risk assessment
 - To self and others
 - Vulnerability/exploitation - finances, relationships
 - Reputation (current employment)
 - Further deterioration
- Mental health act specific - intention to attend the tribuna, support persons such family and legal representation

Q38.3 Medical student asks you – what evidence is there to support use of CTOs/MHA in the management of psychotic illnesses?

- Difficult to study given limited ability to perform randomise studies, as well as ethical and legal concerns
- Evidence is considered limited overall
- Steve Kisely meta-analysis 2014
 - Based on 3 RCTs
 - Did not reduce readmissions or bed days
 - No significant difference in psychiatric symptoms or GAF
 - Less likely to be a victim of crime
- 2016 updated the meta-analysis to include non-RCTs

The patient has worked in a 4 year position as a radio presenter & asks you if you would come into the radio station & do an interview with him about a breaking news story on a recent inpatient discharged who went on to commit a violent murder.

Q38.4 What ethical considerations would you think about before agreeing to or refusing this interview?

- Scope of practice - expertise in forensic matters
- Confidentiality - relating to alleged offender
- Legal implications relating to pending trial. Right to fair trial
- Goldwater rule - not commenting on people that you haven't directly assessed
- Appropriateness of situation. Need for boundaries (with your patient) vs therapeutic alliance
- Non-maleficence and Beneficence - impact on the case/patient rejection vs community education and awareness
- Additional from RANZCP Code of Ethics
 - Sharing knowledge
 - Upholding integrity of the medical profession